

Claim Bundle — GuardianCare Hospital and Surgical Plan

Pacific Guardian Life Assurance (H.K.) Limited
28/F Harbour Centre, 25 Harbour Road, Wan Chai, Hong Kong

Dear Policyholder,

Thank you for notifying us of a hospital admission under your GuardianCare Hospital and Surgical Plan. As requested, we enclose the complete claim bundle. It brings together, in one document, everything you need to lodge the claim and everything we will refer to when we assess it: a summary of the plan, the full policy wording, our claims guide with a specimen of a completed form, the claim form itself, the enclosure list, our standard letters, the schedules of benefits and premiums, the hospital and clinic directories, the complaints procedure, and our personal data statement.

Please complete only the claim form in Section 4. Every other section is for reference and should not be marked. When the form is complete, submit it with the original supporting documents by post, in person at any branch, or through the customer portal as described in Section 3.3. The claim form must reach us within 90 days of the date of discharge.

If you have any question about the bundle or about the claim, please call the hospital claims hotline on 2830 4400 during business hours.

Yours faithfully,

Thomas Reid
Senior Claims Assessor, Hospital Claims Team

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Section 1. Product Summary — GuardianCare Hospital and Surgical Plan

This summary describes the main features of the GuardianCare Hospital and Surgical Plan issued by Pacific Guardian Life Assurance (H.K.) Limited (the Company). It is provided for reference only and does not form part of the contract. The full terms and conditions are set out in the Policy Wording in Section 2 of this bundle. Where this summary and the Policy Wording differ, the Policy Wording prevails.

What the plan is for

The plan reimburses eligible medical expenses incurred by the Insured Person when he or she is confined in a hospital as an inpatient, or receives day-case surgery, for a covered illness or injury. It also reimburses a limited amount of pre-admission and post-discharge outpatient treatment directly connected with the confinement, and provides a fixed daily cash benefit while the Insured Person is confined in a public hospital general ward. The plan is not a comprehensive outpatient plan and does not pay for routine consultations, dental care, or optical care unless they arise from an accident and are stated as covered.

Plan levels

The plan is available at three levels. The level chosen determines the room type that is fully covered, the annual limit, and the premium. All three levels share the same benefit structure; only the amounts differ. A change of level may be requested at each policy anniversary and is subject to underwriting where the change is to a higher level.

Feature	Standard	Superior	Premier
Room type fully covered	General ward	Semi-private	Private
Annual benefit limit	HK\$ 400,000	HK\$ 800,000	HK\$ 1,600,000
Lifetime benefit limit	HK\$ 2,000,000	HK\$ 4,000,000	HK\$ 8,000,000
Deductible per policy year	Nil	Nil	Nil or HK\$ 20,000 (elected)
Pre-admission outpatient window	30 days	30 days	60 days
Post-discharge outpatient window	60 days	90 days	90 days
Daily cash for public general ward	HK\$ 500 per day	HK\$ 800 per day	HK\$ 1,200 per day
Overseas emergency treatment	Not covered	Covered, 90 days per trip	Covered, 180 days per trip
Renewal	Guaranteed to age 80	Guaranteed to age 80	Guaranteed to age 85

Who can apply

Applicants must be aged between 15 days and 65 years at the date of application and must ordinarily reside in Hong Kong. Dependants may be included on the same policy provided they meet the same age and residence requirements. A child dependant is covered until the policy anniversary following the child's twenty-fifth birthday, after which the child may apply for a separate policy without evidence of insurability if the application is received within 60 days.

What is not covered — the main points

- Pre-existing conditions that were not disclosed at application, and any condition arising from them.
- Treatment that is not medically necessary, including confinement for diagnostic tests alone where the tests could reasonably have been performed on an outpatient basis.
- Cosmetic treatment, elective procedures, fertility treatment, and sex reassignment, unless stated as covered.
- Illness arising within the first 30 days after the policy effective date (the waiting period), other than an illness caused by an accident.

- Injury sustained while under the influence of alcohol or drugs, while participating in a hazardous activity listed in the Policy Wording, or as a result of an intentionally self-inflicted act.
- Treatment received outside Hong Kong under the Standard level, and non-emergency treatment received outside Hong Kong under any level.

Making a claim — in brief

Submit the completed claim form together with the original itemised bills, official receipts, the medical certificate or discharge summary, and a copy of the Insured Person's identification document within 90 days of the discharge date. Claims may be lodged by post, in person at any branch, or through the customer portal. The detailed procedure, a checklist, a completed specimen form, and answers to common questions are given in Section 3 of this bundle. The claim form itself is provided in Section 4.

Cooling-off period

A policyholder who is not satisfied with the plan may cancel it within 21 days after the delivery of the policy or the issue of the notice informing the policyholder of the availability of the policy, whichever is earlier, by giving written notice to the Company. Any premium paid will be refunded without interest, provided that no claim has been made.

Section 2. Policy Wording

This Policy is a contract between the Policyholder and Pacific Guardian Life Assurance (H.K.) Limited. It is issued in consideration of the application and the payment of premium. The Company agrees to pay the benefits described in this Policy, subject to its terms, conditions, exclusions and limits. The Policy consists of this Policy Wording, the Policy Schedule, the application, any medical evidence supplied, and any endorsement issued by the Company. Words with a capital initial have the meanings given in Part A.

Part A. Definitions

1. Accident means a sudden, unforeseen and involuntary event caused by external, violent and visible means, occurring at an identifiable time and place, which is the sole and independent cause of Injury.
2. Anniversary Date means the date each year that corresponds to the Policy Effective Date. Where there is no corresponding date in a month, the last day of that month applies.
3. Benefit Limit means the maximum amount payable for a particular benefit as shown in the Schedule of Benefits for the Plan Level in force.
4. Confinement or Confined means an admission to a Hospital as a registered inpatient for a continuous period of not less than six hours on the recommendation of a Physician, or, where a room and board charge is made, for any period. Confinement ends when the Insured Person is discharged.
5. Congenital Condition means a medical, physical or mental abnormality existing at birth, whether or not it is diagnosed or manifests at birth, and any condition that develops as a direct consequence of such an abnormality.
6. Day Case Procedure means a surgical procedure performed in a Hospital or in a licensed day surgery centre where the Insured Person is admitted and discharged on the same day and no room and board charge is made.
7. Deductible means the amount of Eligible Expenses in each Policy Year that the Policyholder has elected to bear before any benefit becomes payable, as shown in the Policy Schedule.
8. Dependant means the spouse of the Policyholder, or an unmarried child of the Policyholder who is aged from 15 days to 25 years and is not in full-time employment, and who is named in the Policy Schedule as an Insured Person.
9. Eligible Expenses means the Reasonable and Customary charges actually incurred for Medically Necessary treatment of a covered Illness or Injury, to the extent that they fall within the benefits and limits of this Policy.
10. Hospital means an institution that is legally licensed as a medical or surgical hospital in the place where it is located, operates primarily for the reception, care and treatment of sick or injured persons as inpatients, provides twenty-four-hour nursing service by registered nurses, has one or more Physicians available at all times, and maintains organised facilities for diagnosis and major surgery. It does not include a clinic, a nursing or convalescent home, a rest home, a place for the care of the aged, a rehabilitation centre, a hospice, or a facility for the treatment of alcoholism or drug addiction, even where such a facility forms part of a Hospital.
11. Illness means a physical condition marked by a pathological deviation from the normal healthy state, which first manifests itself after the Waiting Period and during the Period of Insurance.
12. Injury means bodily injury caused solely and directly by an Accident and independently of any other cause, which occurs during the Period of Insurance.

13. Insured Person means a person named as such in the Policy Schedule, being the Policyholder or a Dependant.
14. Intensive Care Unit means a section of a Hospital that is designated as an intensive care unit by the Hospital, is maintained on a twenty-four-hour basis solely for the treatment of patients in a critical condition, and is equipped to provide special nursing and medical services not available elsewhere in the Hospital.
15. Medically Necessary means a medical service, procedure or supply that is consistent with the diagnosis and customary medical treatment for the condition; is in accordance with standards of good medical practice; is not primarily for the convenience of the Insured Person, the Physician or any other person; and could not have been omitted without adversely affecting the Insured Person's condition or the quality of medical care rendered.
16. Period of Insurance means the period beginning on the Policy Effective Date and ending on the day before the next Anniversary Date, and each subsequent period of one year for which the Policy is renewed.
17. Physician means a person who is legally registered and licensed to practise western medicine in the place where the treatment is given, and who is not the Insured Person, the Policyholder, or a member of their immediate family, business partner, employer or employee.
18. Plan Level means the level of cover (Standard, Superior or Premier) shown in the Policy Schedule.
19. Policy Effective Date means the date shown as such in the Policy Schedule, being the date on which cover under this Policy first begins.
20. Policyholder means the person named as such in the Policy Schedule, who is the owner of the Policy and to whom the Company will pay any benefit unless another payee is validly nominated on the claim form.
21. Pre-existing Condition means any Illness, Injury or condition for which the Insured Person, before the Policy Effective Date, had received medical advice, diagnosis, care or treatment, or had symptoms that would have caused an ordinarily prudent person to seek medical advice, whether or not a diagnosis was made.
22. Reasonable and Customary means, in relation to a charge, a charge that does not exceed the general level of charges made by providers of similar standing in the locality where the charge is incurred, for similar treatment, services or supplies to individuals of the same sex and comparable age, for a similar Illness or Injury.
23. Room and Board means the daily charge for accommodation and general nursing care in a Hospital, excluding charges for medical services, drugs, dressings and the use of special facilities.
24. Schedule of Benefits means the table of benefits and limits that forms part of this Policy and is reproduced in Section 7 of this bundle for reference.
25. Semi-private Room means a room in a Hospital with two or more beds, accommodating two patients, and where the Hospital's semi-private rate applies.
26. Surgery means an operation performed by a Physician for the treatment of an Illness or Injury, including the necessary anaesthesia, and including endoscopic procedures where an incision or the passage of an instrument through a natural opening is made for a therapeutic purpose.
27. Waiting Period means the period of 30 days beginning on the Policy Effective Date, or on the date of reinstatement where the Policy has lapsed and been reinstated, during which no benefit is payable for Illness.

Part B. Benefits

Subject to the terms of this Policy, the Company will reimburse the Eligible Expenses incurred by an Insured Person during the Period of Insurance for the items listed in this Part, up to the Benefit Limits shown in the Schedule of Benefits and subject to the Annual and Lifetime Benefit Limits. Any Deductible elected by the Policyholder is applied once in each Policy Year to the total Eligible Expenses of that Policy Year before benefit is calculated.

B1. Hospital confinement benefits

1. Room and Board. The daily charge for Room and Board during Confinement, up to the daily limit and the maximum number of days per Policy Year shown in the Schedule of Benefits. Where the Insured Person occupies a room of a higher type than that covered under the Plan Level, the benefits under this Part B1 and Part B2 are reduced in accordance with General Condition C12 (Room type adjustment).
2. Intensive care. The daily charge for accommodation and nursing in an Intensive Care Unit, up to the daily limit and the maximum number of days shown in the Schedule of Benefits. Days of intensive care are counted separately from days of Room and Board.
3. Attending Physician's visits. The charge made by the attending Physician for visits during Confinement, up to one visit per day and the daily limit shown in the Schedule of Benefits.
4. Specialist consultation. The charge made by a specialist Physician for consultation during Confinement, where the consultation is recommended in writing by the attending Physician, up to the limit shown in the Schedule of Benefits.
5. Hospital services. Charges made by the Hospital during Confinement for drugs and medicines prescribed by a Physician, dressings, laboratory examinations, X-ray and other diagnostic imaging, physiotherapy prescribed by a Physician, blood and plasma, and the use of the operating theatre and its equipment, up to the limit shown in the Schedule of Benefits.
6. Private nursing. The charge for the services of a registered nurse engaged on the written recommendation of the attending Physician during Confinement, up to the daily limit and the maximum number of days shown in the Schedule of Benefits.

B2. Surgical benefits

1. Surgeon's fee. The fee charged by the Physician who performs Surgery during Confinement or as a Day Case Procedure, up to the limit for the procedure shown in the Surgical Schedule. Where a procedure is not listed, the Company will determine the limit by reference to a listed procedure of comparable complexity.
2. Anaesthetist's fee. The fee charged by the Physician who administers anaesthesia for covered Surgery, up to 35 percent of the surgeon's fee payable.
3. Operating theatre charge. The charge made by the Hospital or day surgery centre for the use of the operating theatre and recovery room, up to 35 percent of the surgeon's fee payable.
4. Two or more procedures. Where two or more surgical procedures are performed through the same incision during one operating session, the Company will pay for the procedure with the highest limit only. Where the procedures are performed through separate incisions, the Company will pay for the procedure with the highest limit in full and for each additional procedure at 50 percent of its limit.

B3. Pre-admission and post-discharge benefits

1. Pre-admission outpatient. Charges for consultation, diagnostic tests and prescribed medication received on an outpatient basis within the number of days before Confinement shown in the

Schedule of Benefits, provided that the treatment relates directly to the condition for which the Insured Person is subsequently Confined.

2. Post-discharge outpatient. Charges for follow-up consultation, diagnostic tests, prescribed medication, physiotherapy and dressing received on an outpatient basis within the number of days after discharge shown in the Schedule of Benefits, provided that the treatment relates directly to the condition for which the Insured Person was Confined.
3. Home nursing. Where the attending Physician certifies in writing that the services of a registered nurse at the Insured Person's home are Medically Necessary immediately following discharge, the charge for such services up to the daily limit and the maximum number of days shown in the Schedule of Benefits.

B4. Other benefits

1. Public hospital daily cash. Where the Insured Person is Confined in a general ward of a public Hospital in Hong Kong and no charge other than the standard public ward charge is incurred, the Company will pay the daily cash amount shown in the Schedule of Benefits for each day of Confinement, up to the maximum number of days per Policy Year, in lieu of all other benefits under Parts B1 to B3 for that Confinement.
2. Accidental emergency outpatient. Charges for emergency outpatient treatment received in a Hospital within 24 hours of an Accident, up to the limit shown in the Schedule of Benefits, whether or not Confinement follows.
3. Emergency dental. Charges for treatment by a registered dentist to restore sound natural teeth damaged in an Accident, received within 14 days of the Accident, up to the limit shown in the Schedule of Benefits.
4. Ambulance. The charge for ambulance transport to or from a Hospital where Medically Necessary, up to the limit shown in the Schedule of Benefits.
5. Companion bed. Where the Insured Person is a child under 12 years of age, the charge made by the Hospital for a bed for one parent or guardian to stay in the same room, up to the daily limit and the maximum number of days shown in the Schedule of Benefits.
6. Compassionate death benefit. On the death of the Insured Person from a covered Illness or Injury during the Period of Insurance, the Company will pay the fixed amount shown in the Schedule of Benefits to the Policyholder or, if the Policyholder is the deceased, to the Policyholder's estate.
7. Overseas emergency treatment. Under the Superior and Premier Plan Levels only, charges for emergency Confinement outside Hong Kong that begins within the number of days of departure from Hong Kong shown in the Schedule of Benefits, for a condition that arose during the trip and could not reasonably have been foreseen. Benefits are paid on the same basis as for Confinement in Hong Kong, and the Company may require the Insured Person to be repatriated to Hong Kong for continued treatment where the attending Physician certifies that repatriation is medically safe.

Part C. Exclusions

This Policy does not cover, and the Company will not pay any benefit in respect of, any charge, expense, loss or liability arising directly or indirectly from, or in connection with, any of the following.

1. Any Pre-existing Condition, unless it was fully disclosed in the application and accepted by the Company in writing without exclusion.
2. Any Illness that first manifests itself during the Waiting Period, and any consequence of it.
3. Treatment that is not Medically Necessary, or that is not recommended by a Physician, or that is received from a person who is not a Physician within the meaning of this Policy.
4. Confinement primarily for diagnostic procedures, X-ray examinations, physical therapy, rehabilitation, convalescence, custodial care or rest, where such services could reasonably have been provided without Confinement.
5. Routine medical examinations, health checks, vaccinations, screening tests, and any treatment or investigation that is not incidental to the treatment of a covered Illness or Injury.
6. Cosmetic or plastic surgery, unless it is reconstructive surgery Medically Necessary as a result of an Accident that occurred during the Period of Insurance and is performed within 12 months of that Accident.
7. Dental treatment and oral surgery, other than the emergency dental benefit under Part B4, and any treatment relating to dentures, crowns, bridges or implants.
8. Eye refraction, the fitting of spectacles or contact lenses, and any procedure to correct refractive error of the eye.
9. Pregnancy, childbirth, miscarriage, abortion and their complications; infertility, contraception, sterilisation and any form of assisted reproduction; and any treatment of sexual dysfunction.
10. Congenital Conditions and hereditary conditions, and any developmental condition of a child, whether or not diagnosed at birth.
11. Mental illness, psychiatric disorders, behavioural disorders, and any treatment by a psychiatrist or psychologist, unless stated as covered by an endorsement.
12. Alcoholism, drug addiction, substance abuse, and any Illness or Injury sustained while the Insured Person was under the influence of alcohol or of any drug not prescribed by a Physician.
13. Intentionally self-inflicted injury, suicide or attempted suicide, whether the Insured Person was sane or insane.
14. Human immunodeficiency virus (HIV) infection and any condition related to it, including acquired immune deficiency syndrome (AIDS), unless the infection was acquired through a blood transfusion received in Hong Kong during the Period of Insurance.
15. Sexually transmitted diseases and their consequences.
16. War, invasion, act of foreign enemy, hostilities (whether war is declared or not), civil war, rebellion, revolution, insurrection, military or usurped power, riot, strike, civil commotion, and terrorism.
17. Nuclear fission, nuclear fusion, ionising radiation, radioactive contamination, and the use of any chemical or biological agent as a weapon.
18. Participation in any professional sport; in any race or speed contest other than on foot; in mountaineering requiring the use of ropes or guides; in scuba diving to a depth greater than 30 metres; in parachuting, hang-gliding, paragliding, bungee jumping, or any aerial activity other than as a fare-paying passenger on a licensed commercial aircraft.
19. Service in any armed force, and any activity undertaken while on military duty.

20. Any criminal act committed by the Insured Person, and any Injury sustained while resisting lawful arrest.
21. Experimental treatment, treatment that has not been proven effective by generally accepted medical standards, and any drug or device that has not been approved for the relevant use by the appropriate regulatory authority.
22. Non-medical items, including telephone, television, newspaper, guest meals, personal comfort items, and charges for a room type or service that is not Medically Necessary.
23. Charges incurred after the expiry or termination of this Policy, other than charges for a Confinement that began before termination and continues without interruption, which will be covered for not more than 30 days after termination.
24. Any charge for which the Insured Person is entitled to reimbursement from any other source, to the extent of that reimbursement, in accordance with General Condition C11 (Other insurance).

Part D. General Conditions

1. Entire contract. This Policy, the Policy Schedule, the application, any declaration or statement made in connection with the application, and any endorsement together constitute the entire contract. No agent or representative of the Company has authority to vary any term of this Policy or to waive any of its provisions; a variation is effective only if it is set out in an endorsement signed by an authorised officer of the Company.
2. Duty of disclosure. The Policyholder and each Insured Person must disclose to the Company every fact that they know, or could reasonably be expected to know, that would influence the judgement of a prudent insurer in deciding whether to accept the risk and on what terms. This duty continues until the Policy is issued and applies again on each renewal, reinstatement, and application for a change of Plan Level.
3. Misstatement of age. If the age of an Insured Person has been misstated, the premium will be adjusted to the correct age, and any benefit will be that which the premium actually paid would have purchased at the correct age. If the correct age would have made the Insured Person ineligible, the Company's liability is limited to a refund of the premium paid for that Insured Person without interest.
4. Incontestability. After this Policy has been in force for two years from the Policy Effective Date, or from the date of the last reinstatement, the Company will not contest its validity except for fraud or non-payment of premium.
5. Premium. The premium is payable in advance on the Policy Effective Date and on each subsequent due date shown in the Policy Schedule. Premium rates are not guaranteed and may be revised by the Company at any Anniversary Date by giving not less than 30 days' written notice. Premiums vary with the attained age of the Insured Person and are set out in the premium tables published by the Company from time to time.
6. Grace period. A grace period of 31 days is allowed for the payment of each premium after the first, during which the Policy remains in force. If the premium is not paid within the grace period, the Policy lapses on the due date. Any benefit that becomes payable during the grace period will be paid after deduction of the outstanding premium.
7. Reinstatement. A Policy that has lapsed may be reinstated within 12 months of the lapse date at the Company's discretion, on payment of all overdue premiums and provision of evidence of insurability satisfactory to the Company. On reinstatement, a new Waiting Period applies to Illness, and the Company is liable only for Illness that first manifests, or Injury that occurs, after the reinstatement date.
8. Renewal. Subject to the age limit for the Plan Level, this Policy is guaranteed renewable on each Anniversary Date on payment of the premium then applicable, regardless of the claims experience or health of the Insured Person. The Company reserves the right to revise the terms, benefits and premiums of the plan for all policies of the same class at renewal.
9. Termination. Cover for an Insured Person terminates on the earliest of: the date the Policy lapses or is cancelled; the Anniversary Date following the Insured Person's attainment of the maximum age for the Plan Level; the date the Insured Person ceases to be a Dependant; the date the Insured Person ceases to ordinarily reside in Hong Kong for more than 183 days in a Policy Year; and the date of the Insured Person's death.
10. Cancellation. The Policyholder may cancel this Policy at any time by written notice. Where notice is received after the cooling-off period, cancellation takes effect on the next premium due date and no refund is made for the remaining period. The Company may cancel this Policy for fraud, non-disclosure, or non-payment of premium by giving 30 days' written notice to the Policyholder's last known address.

11. Other insurance. Where the Insured Person is entitled to reimbursement of any Eligible Expenses from any other insurance policy, medical scheme, employer benefit or statutory source, the Company will pay only the amount by which the Eligible Expenses exceed the amount recoverable from that other source. The Policyholder must disclose all other cover when making a claim.
12. Room type adjustment. Where the Insured Person occupies a room of a higher type than that covered under the Plan Level, all benefits under Parts B1 and B2 for that Confinement are reduced to the proportion that the Room and Board limit for the covered room type bears to the actual Room and Board charge incurred, except where the higher room type was the only accommodation available and the Hospital confirms this in writing.
13. Currency. All amounts under this Policy are expressed and payable in Hong Kong dollars. Expenses incurred in another currency are converted at the Company's prevailing rate of exchange on the date of the last day of treatment.
14. Assignment. No assignment of this Policy or of any benefit under it is binding on the Company unless it is in writing and has been recorded by the Company. The Company assumes no responsibility for the validity of any assignment.
15. Notices. Every notice to the Company must be in writing and delivered to the Company's principal office in Hong Kong. Every notice to the Policyholder is deemed to have been received five days after posting to the last address notified to the Company, or on the day of transmission where sent electronically to a notified address.
16. Governing law. This Policy is governed by the laws of the Hong Kong Special Administrative Region, and the courts of Hong Kong have exclusive jurisdiction over any dispute arising from it.
17. Third party rights. A person who is not a party to this Policy has no right under the Contracts (Rights of Third Parties) Ordinance to enforce any of its terms.
18. Sanctions. The Company will not provide cover, pay any claim or provide any benefit to the extent that doing so would expose the Company to any sanction, prohibition or restriction under United Nations resolutions or the trade or economic sanctions, laws or regulations of Hong Kong, the European Union, the United Kingdom or the United States of America.

Part E. Claims Conditions

1. Notice of claim. Written notice of a claim must be given to the Company within 30 days of the commencement of Confinement, or as soon as reasonably practicable thereafter. Failure to give notice within this period will not invalidate the claim if it is shown that notice was given as soon as was reasonably possible.
2. Proof of loss. The completed claim form and all supporting documents must be received by the Company within 90 days of the date of discharge or, for outpatient benefits, of the date of the last treatment. The Company may in its discretion accept a late claim where a written explanation of the delay is provided and the delay has not prejudiced the Company's assessment.
3. Supporting documents. Every claim must be accompanied by the original itemised bills and official receipts showing the name of the Insured Person, the date and nature of each item, and the amount charged; a medical certificate or discharge summary signed by the attending Physician stating the diagnosis, the dates of Confinement and the treatment given; a copy of the Insured Person's identification document; and, where the claim arises from an accident, a police report or other independent evidence of the accident where available. Photocopies of receipts are accepted only where the original has been submitted to another insurer, in which case that insurer's settlement advice must also be provided.
4. Medical examination. The Company may, at its own expense, require the Insured Person to be examined by a Physician nominated by the Company as often as is reasonably necessary while a claim is pending, and may require a post-mortem examination in the case of death where this is not prohibited by law.
5. Access to records. The Insured Person authorises any Hospital, Physician or other person who has attended the Insured Person to release to the Company all information relating to the claim, and agrees to sign any further authorisation the Company may reasonably require for this purpose.
6. Payment of claims. Benefits are payable to the Policyholder, or to the payee nominated on the claim form where the Policyholder has signed the nomination, or to the Policyholder's estate. Payment by bank transfer is made to the account stated on the claim form; the Company accepts no liability for payment to an account whose details were incorrectly stated by the claimant. Payment by cheque is made to the payee's name as stated on the claim form and is posted to the correspondence address on file.
7. Time of payment. The Company will pay a claim within 10 working days of receiving all documents necessary to assess it. Where further information is required, the Company will write to the claimant within 5 working days of receipt of the claim form setting out what is required.
8. Fraudulent claims. If a claim is in any respect fraudulent, or if any fraudulent means or device is used by the Policyholder, the Insured Person or anyone acting on their behalf to obtain benefit under this Policy, all benefit under this Policy in respect of that claim is forfeited, the Company may cancel this Policy from the date of the fraudulent act, and the Company may recover any benefit already paid.
9. Recovery. Where a benefit has been paid and it is later established that the claim was not covered, or that the amount paid exceeded the amount properly payable, the Policyholder must repay the excess to the Company on demand, and the Company may set off the amount against any other benefit payable.
10. Legal action. No action at law or in equity may be brought to recover under this Policy before the expiry of 60 days after proof of loss has been furnished in accordance with these conditions, nor after the expiry of two years from the date on which proof of loss was required to be furnished.

11. Arbitration. Any dispute concerning the amount payable under this Policy, where liability is otherwise admitted, shall be referred to the decision of a single arbitrator to be agreed by the parties or, failing agreement within 30 days, to be appointed by the Chairman of the Hong Kong International Arbitration Centre. The making of an award shall be a condition precedent to any right of action against the Company.

Supplement A -- Optional Benefit Riders: Policy Wording

This Supplement sets out the terms of four optional benefit riders that may be attached to the GuardianCare Hospital and Surgical Plan: the Dental Care Rider, the Maternity Rider, the Outpatient Top-Up Rider, and the Critical Illness Lump-Sum Rider (each a Rider). A Rider forms part of this Policy only where it is shown as attached, with an effective date, in the Policy Schedule. Where no Rider is shown as attached, the material in this Supplement does not apply and may be disregarded for the purposes of the Insured Person's cover. Expressions defined in Part A of the Policy Wording carry the same meaning in this Supplement. This Supplement adds further defined terms that apply only to the Rider to which they relate, as set out under each heading below.

A.1 General Provisions Applicable to All Riders

The four Riders described in this Supplement share a common structure. Each is optional, each carries its own premium, and each is subject to its own waiting period, benefit limit and exclusions in addition to the terms of the main Policy. A Rider does not widen the definition of Illness or Injury used elsewhere in this Policy, and it does not remove an exclusion in Part C of the Policy Wording except to the extent expressly stated under the heading for that Rider.

1. Rider means an optional benefit described in this Supplement that is shown as attached to this Policy in the Policy Schedule, together with its effective date and the Plan Level or amount to which it applies.
2. Rider Effective Date means the date from which cover under a particular Rider begins, as shown in the Policy Schedule. Where a Rider is added after the Policy Effective Date, its Rider Effective Date is the date the endorsement adding it takes effect, and not the Policy Effective Date.
3. Rider Premium means the additional premium payable for a Rider, shown in the Policy Schedule, which is payable together with, and on the same payment mode as, the premium for this Policy.
4. Rider Schedule means the part of the Policy Schedule, or an endorsement, that records which Riders are attached, their effective dates, and the benefit amounts or limits that apply to each of them.
5. Rider Year means each period of twelve months beginning on a Rider Effective Date, or on an Anniversary Date on or after which the Rider remains attached, used to measure an annual limit or an annual number of visits or sessions under a Rider.

A Rider may be attached at the Policy Effective Date, in which case it is recorded in the original Policy Schedule, or it may be added at a later Anniversary Date by written application. An application to add a Rider after the Policy Effective Date is treated as a fresh application for that Rider and is subject to underwriting, including, where the Company considers it necessary, medical evidence relating to the proposed Insured Person. The Company may accept the application, decline it, or accept it subject to a special exclusion or a loading on the Rider Premium, and will confirm the outcome in writing before the Rider Effective Date.

1. A Rider cannot be held on its own. It attaches to, and takes effect only while, this Policy is in force for the Insured Person named in the Rider Schedule.
2. Where a Rider is added after the Policy Effective Date, the waiting period for that Rider described under the relevant heading below runs from the Rider Effective Date shown for that addition, and not from the original Policy Effective Date.
3. The Rider Premium is shown separately from the premium for the main Policy in each renewal notice, so that the Policyholder can identify the cost of each Rider that is attached.

4. Where the Plan Level of the main Policy changes at an Anniversary Date, a Rider benefit that is expressed as a proportion of, or by reference to, the main Policy's benefit amount changes automatically to match the new Plan Level, unless the Rider Schedule states a fixed amount that does not vary with Plan Level.
5. A request to add a Rider does not affect the terms already in force for the main Policy or for any other Rider already attached.

Because each Rider adds its own waiting period, the table below sets out, for quick reference, the waiting period applicable to a newly attached Rider and the further condition that applies before a first claim can be made under it. The full wording of each waiting period is set out under the relevant heading that follows.

Rider	Waiting period from Rider Effective Date	Further condition before a first claim
Dental Care Rider	30 days for routine treatment; 12 months for restorative treatment	None beyond the waiting period itself
Maternity Rider	10 months for delivery-related benefits	24 hours only where a complication is caused by an Injury
Outpatient Top-Up Rider	30 days	Annual visit cap applies from the first Rider Year
Critical Illness Lump-Sum Rider	90 days	A Survival Period of 14 days after the Diagnosis Date

The remainder of this Supplement sets out the cover, the conditions and the exclusions that apply to each Rider in turn. A reference in this Supplement to "this Rider" means the Rider described under the heading in which the reference appears, and a reference to a Part or a Section without more is a reference to that Part or Section of the Policy Wording or the bundle in which this Supplement is bound.

A benefit payable under a Rider is calculated, and paid, in the same currency as the main Policy. Where a Rider Benefit Limit is expressed as an amount rather than as a proportion of a main Policy benefit, that amount is not increased or reduced by movement in the exchange rate between the currency in which an expense was incurred and the currency of this Policy; the conversion is carried out at the rate the Company applies generally to claims under Part E of the Policy Wording, on the date the expense was incurred.

1. A Rider Benefit Limit is separate from, and does not draw on, the Annual Benefit Limit or the Lifetime Benefit Limit that applies to the main Policy, unless the Rider Schedule expressly states that a Rider benefit is included within one of those limits.
2. Where the Policyholder holds more than one Rider of the same kind for different Insured Persons, each Insured Person has their own separate Rider Benefit Limit for each Rider Year; the limits are not shared between Insured Persons and are not pooled at the level of the policy as a whole.
3. A Rider is not available under this Policy for an Insured Person who is a Dependant not yet born or not yet named in the Policy Schedule; a Rider must be applied for, and accepted, in respect of an Insured Person who is already named before the Rider Effective Date for that person can begin.

An application to add a Rider, whether at the Policy Effective Date or later, is assessed on the medical and other evidence the Company requests for that purpose; the standard evidence requirements are the same as those described for an application for the main Policy, adjusted where necessary to the particular Rider applied for. For the Maternity Rider, an application received after a pregnancy has already begun is declined; for the Critical Illness Lump-Sum Rider, an application received after a Critical Illness has already been diagnosed, whether or not the Insured Person was aware of the diagnosis at the time of applying, is treated as a non-disclosure under Part D of the Policy Wording if it is later discovered.

Where the Policyholder changes the Plan Level of the main Policy at an Anniversary Date, a Rider already attached does not need to be reapplied for; it continues on its existing Rider Effective Date and its existing waiting period is not affected by the change of Plan Level. Where the Plan Level change is a reduction, a Rider Benefit Limit expressed as a fixed amount in the Rider Schedule is unaffected, but a Rider benefit that is instead expressed by reference to a main Policy benefit, such as the daily Room and Board limit used to calculate the benefit described in A.3 below, is recalculated using the reduced Plan Level from the Anniversary Date on which the reduction takes effect.

A Rider benefit is payable to the Policyholder in the same way as a benefit under the main Policy, unless the Policyholder has validly nominated a different payee on the claim form for the particular claim; a nomination made for a claim under the main Policy does not automatically apply to a claim under a Rider, and a separate nomination should be made on each claim form where the Policyholder wishes a Rider benefit to be paid to someone else. Nothing in this Supplement changes the Company's right, described in Part E of the Policy Wording, to require further evidence of entitlement to receive payment before a benefit, whether under the main Policy or under a Rider, is released.

Where a Rider is attached, the Policy Schedule records it by an endorsement number distinct from the main Policy's certificate number, together with the Rider Effective Date and, where relevant, any special exclusion or loading agreed at the time it was added. An Insured Person who wishes to confirm which Riders are currently attached, and on what terms, should refer to the current Policy Schedule rather than to an earlier renewal notice, since a change agreed after a renewal notice was issued is recorded on the Policy Schedule itself and on the confirmation letter that accompanies the change, and not on the notice that had already been sent before the change was agreed.

A.2 Dental Care Rider -- Policy Wording

This Rider provides a benefit towards the cost of dental treatment received from an Attending Dentist. It is not a comprehensive dental plan: routine treatment is covered up to modest annual limits, and restorative and surgical treatment is covered mainly where it follows an Injury to a sound natural tooth. Where this Rider is attached, it overrides the general exclusion of dental treatment in Part C of the Policy Wording only to the extent described below.

1. Attending Dentist means a person who is legally registered and licensed to practise dentistry in the place where the treatment is given, and who is not the Insured Person or a member of their immediate family.
2. Dental Treatment means treatment given by an Attending Dentist for the examination, prevention or treatment of a condition of the teeth, gums or supporting structures, but does not include Orthodontic Treatment or treatment described as excluded below.
3. Orthodontic Treatment means treatment intended to correct the position or alignment of the teeth, including the fitting of braces, retainers or aligners, whether or not the treatment also has a restorative purpose.
4. Restorative Treatment means treatment to repair or replace a Sound Natural Tooth that has been damaged, including a filling, a crown, an inlay, a bridge, or the extraction and replacement of the tooth, but does not include treatment for a tooth that was already decayed, loose, or previously restored before the damage occurred.
5. Sound Natural Tooth means an Insured Person's own tooth that is free of decay, is not more than minimally restored, and is not itself a cause of the condition for which treatment is later required.

Subject to the annual limit shown in the Rider Schedule, the Company will reimburse the Eligible Expenses of the following treatment given by an Attending Dentist during a Rider Year.

1. Consultation and examination, including a routine check-up, up to twice in each Rider Year.

2. Scaling and polishing, up to twice in each Rider Year.
3. A filling to treat decay, and an extraction where an Attending Dentist certifies that the tooth cannot reasonably be preserved.
4. Root canal treatment, where an Attending Dentist certifies in writing that it is necessary to preserve the tooth.
5. An intra-oral X-ray or a panoramic X-ray taken in connection with treatment otherwise covered under this Rider.
6. Restorative Treatment to a Sound Natural Tooth damaged by an Injury, provided treatment begins within 90 days of the Injury; this benefit is payable even where the general exclusion of dental treatment in Part C would otherwise apply, but remains subject to the annual limit shown in the Rider Schedule.

This Rider is subject to a waiting period of 30 days from the Rider Effective Date for the benefits described in items 1 to 3 of the Cover above, and 12 months from the Rider Effective Date for the benefits described in items 4 and 5. The waiting period does not apply to the benefit described in item 6, which is payable from the Rider Effective Date provided the Injury occurs after that date. Treatment must be completed, and the account settled, within the Rider Year in which it is given for it to be reimbursed under that Rider Year's limit; treatment carried across two Rider Years is apportioned between them by reference to the date each part of the treatment was given.

- Orthodontic Treatment of any kind, and any treatment to correct malocclusion, whether or not the malocclusion is congenital.
- Implants, and any treatment to prepare for or maintain an implant, other than replacement of a Sound Natural Tooth lost to an Injury as described above.
- Treatment that is primarily cosmetic, including tooth whitening or bleaching, veneers applied for appearance alone, and any treatment not shown to be necessary for the health of the tooth or its supporting structure.
- Treatment for a condition that was already being investigated or treated before the Rider Effective Date, or for a tooth already identified as requiring the treatment before that date.
- Treatment received from a person who is not an Attending Dentist, or treatment given outside a properly equipped dental surgery.
- Replacement of a denture, bridge or crown that is lost or damaged through the Insured Person's own neglect, or that could reasonably have continued in use.

Where pain relief is required outside the Attending Dentist's normal consulting hours, emergency treatment to relieve acute dental pain is reimbursed under item 3 of the Cover above regardless of the annual visit count otherwise applying to a routine check-up, provided the treatment given is limited to what is needed to relieve the pain and to stabilise the tooth until a further appointment can be arranged. A specialist referral for oral surgery, including the surgical removal of an impacted tooth, is reimbursed under this Rider only where the impacted tooth is causing a diagnosed infection or is directly damaging an adjacent Sound Natural Tooth; the removal of an impacted tooth for preventive reasons alone is not covered.

The annual limit shown in the Rider Schedule for this Rider does not carry forward: any part of the limit not used in a Rider Year is not added to the limit available in the following Rider Year, and a claim cannot be brought forward from a later Rider Year to make use of an earlier Rider Year's unused limit. Where treatment recommended by an Attending Dentist would exceed the annual limit if carried out in a single Rider Year, the Attending Dentist may, where clinically appropriate, phase the treatment across two Rider Years, and each phase is then assessed against the limit for the Rider Year in which it is given.

Where an Attending Dentist recommends root canal treatment described in item 4 of the Cover above and also recommends that a specialist endodontist carry it out, the specialist's fee is reimbursed on the same basis as if the Attending Dentist had carried out the treatment personally, up to the same limit; a referral of this kind does not require separate approval from the Company before the treatment is given, although the Company may ask the Attending Dentist to confirm in writing why a referral to a specialist was clinically appropriate where the fee claimed is materially higher than the usual charge for the same procedure.

A.3 Maternity Rider -- Policy Wording

This Rider provides a benefit towards the cost of a Delivery and towards certain complications connected with pregnancy. It does not convert this Policy into a maternity plan of general application: benefits are limited to those described below, and antenatal care that is not connected with a covered Delivery or a Complication of Pregnancy remains excluded.

1. Delivery means the birth of a child, by any method, after the completion of at least 24 weeks of pregnancy.
2. Complication of Pregnancy means a condition, distinct from a normal pregnancy, that requires treatment before Delivery and that, if untreated, would endanger the life of the mother or the unborn child, including an ectopic pregnancy, a molar pregnancy, pre-eclampsia requiring admission, and antepartum haemorrhage requiring admission.
3. Caesarean Delivery means a Delivery by surgical operation through an incision in the abdominal wall and uterus.
4. Newborn Child means a child born of a Delivery for which a benefit is payable under this Rider, for the period described in the Conditions below.
5. Rider Year, for the purposes of this Rider, has the meaning given in A.1 above, save that where a Delivery occurs, the Rider Year in which the benefit for that Delivery is payable is the Rider Year current at the date of the Delivery.
6. Confinement, for the purposes of this Rider, includes a stay in a Hospital for the purpose of Delivery even where none of the other conditions in the general definition of Confinement in Part A of the Policy Wording is separately met.

Where this Rider is attached and has been in force for the qualifying period described in the Conditions below, the exclusion of pregnancy, childbirth and their complications in Part C of the Policy Wording does not apply to the extent that a benefit is payable under this Rider.

1. A fixed benefit for a Delivery other than by Caesarean Delivery, up to the amount shown in the Rider Schedule.
2. A higher fixed benefit for a Caesarean Delivery performed on medical grounds certified by the attending Physician, up to the amount shown in the Rider Schedule.
3. Room and Board and the attending Physician's fee for the Confinement in which the Delivery takes place, up to the daily limit and the maximum number of days shown in the Rider Schedule, in place of, and not in addition to, the benefit described in item 1 or item 2 above.
4. Treatment of a Complication of Pregnancy, on the same basis as treatment of an Illness under Part B of the Policy Wording, up to the limit shown in the Rider Schedule for this benefit.
5. Care of a Newborn Child for a Congenital Condition identified within the period described in the Conditions below, up to the limit shown in the Rider Schedule.
6. A course of antenatal check-ups directly connected with a Delivery for which a benefit is paid under item 1 or item 2 above, up to the number of visits shown in the Rider Schedule.

This Rider is subject to a waiting period of 10 months from the Rider Effective Date before a benefit becomes payable under items 1, 2, 3 or 6 of the Cover above. Treatment of a Complication of Pregnancy under item 4 is instead subject to a waiting period of 30 days from the Rider Effective Date, save that this shorter waiting period does not apply, and no waiting period applies at all, where the Complication of Pregnancy is caused by an Injury occurring after the Rider Effective Date. The benefit for care of a Newborn Child under item 5 is payable only for treatment given within 30 days of birth, and only where the Delivery itself was one for which a benefit was payable under this Rider. Only one Delivery benefit is payable under items 1 to 3 in respect of the same pregnancy, regardless of the number of children born of that Delivery.

- Treatment connected with fertility, assisted reproduction, or a pregnancy achieved through assisted reproduction, which remains excluded under Part C of the Policy Wording notwithstanding this Rider.
- A Delivery, or treatment of a Complication of Pregnancy, occurring before the applicable waiting period described above has been satisfied.
- Antenatal care that is not directly connected with a Delivery for which a benefit is paid, including routine check-ups during a pregnancy that ends before the waiting period is satisfied.
- Elective termination of a pregnancy, other than a termination that a Physician certifies is necessary to preserve the life of the mother, which is treated instead as a Complication of Pregnancy.
- Care of a Newborn Child beyond the period described in the Conditions above, or care that is not connected with a Congenital Condition identified within that period.
- Delivery outside a Hospital, other than where the Delivery begins unexpectedly and the Insured Person is admitted to a Hospital as soon as reasonably practicable afterwards for the birth to be completed or for post-delivery care.

Where a Delivery results in more than one child being born, the fixed benefit described in item 1 or item 2 of the Cover above is payable once only for that Delivery, and is not multiplied by the number of children born; the benefit for care of a Newborn Child under item 5, by contrast, is assessed separately for each child, subject to the overall limit shown in the Rider Schedule for that benefit in a Rider Year. A genetic or ultrasound screening test recommended in writing by the attending Physician because the pregnancy has been assessed as high risk is reimbursed as part of the antenatal check-ups described in item 6 of the Cover above, up to the limit shown in the Rider Schedule for that benefit; a screening test taken as a matter of routine, without such a recommendation, is not separately reimbursed.

This Rider does not cover the ongoing care of a mental health condition that first arises after Delivery, including a mood disturbance connected with childbirth, which is assessed instead, if at all, under the general terms of the main Policy or under the Outpatient Top-Up Rider where that Rider is also attached; this Rider does, however, cover a Complication of Pregnancy that has a physical rather than a purely psychological basis, on the same terms as any other Complication of Pregnancy described above. Support for breastfeeding, including the services of a lactation consultant, is not covered under this Rider, whether given during the Confinement for Delivery or afterwards.

Where a Caesarean Delivery is requested by the Insured Person without a medical indication that the attending Physician would otherwise regard as calling for one, the Delivery is treated for the purpose of item 1 or item 2 of the Cover above as a Delivery other than by Caesarean Delivery, and the lower fixed benefit applies; the balance of the account is payable by the Insured Person. Where the attending Physician certifies in writing, before the Delivery, that a Caesarean Delivery is medically indicated for a reason connected with the mother's or the child's condition, the higher fixed benefit under item 2 applies regardless of whether the Insured Person had earlier expressed a preference either way.

A.4 Outpatient Top-Up Rider -- Policy Wording

The main Policy reimburses outpatient treatment only where it is connected with a covered Confinement, within the pre-admission and post-discharge windows described in Part B3 of the Policy Wording. This Rider adds a separate annual benefit for outpatient consultation and related treatment that is not connected with a Confinement, so that routine or minor outpatient treatment does not have to wait for, or depend on, an inpatient episode.

1. Panel Doctor means a Physician, or a clinic, who has agreed with the Company to provide outpatient treatment to Insured Persons under this Rider on agreed terms, and whose name appears in the directory referred to elsewhere in this bundle.
2. Non-panel Doctor means a Physician who is not a Panel Doctor.
3. Visit means a single consultation with a Panel Doctor or a Non-panel Doctor on one day, including any treatment given, medication dispensed, or investigation ordered at that consultation.
4. Referral means a written recommendation from a Panel Doctor for the Insured Person to consult a specialist, without which a consultation with a specialist is treated as a Visit to a Non-panel Doctor for the purpose of the co-payment described in the Conditions below.
5. Specified Investigation means an X-ray, an ultrasound scan, a basic blood test, or a similar outpatient diagnostic test ordered at a Visit, but does not include advanced imaging such as computed tomography or magnetic resonance imaging, which remains subject to the limit in Part B3 of the Policy Wording where connected with a Confinement, and is not covered under this Rider where it is not.

Subject to the annual visit limit and the per-visit limit shown in the Rider Schedule, the Company will reimburse the Eligible Expenses of a Visit, including medication prescribed at that Visit for a course of not more than 14 days, and a Specified Investigation ordered at that Visit. A Visit to a specialist made on a Referral is reimbursed on the same basis as a Visit to a Panel Doctor. Physiotherapy prescribed in writing by a Physician is reimbursed up to the number of sessions shown in the Rider Schedule, counted separately from the annual visit limit.

1. A Visit to a Panel Doctor is reimbursed in full up to the per-visit limit shown in the Rider Schedule.
2. A Visit to a Non-panel Doctor, or a Visit to a specialist made without a Referral, is reimbursed up to 70 percent of the per-visit limit shown in the Rider Schedule, the remainder being borne by the Insured Person.
3. The annual visit limit shown in the Rider Schedule applies to the combined total of Visits to Panel Doctors and Non-panel Doctors in a Rider Year; it is not doubled by using both.
4. Where a Visit is connected with a Confinement for which pre-admission or post-discharge outpatient treatment is separately payable under Part B3 of the Policy Wording, that Visit is paid under Part B3 and not under this Rider, so that the same treatment is not counted twice.
5. This Rider is subject to a waiting period of 30 days from the Rider Effective Date.
 - Routine health screening, vaccination, and examinations required for employment, travel, insurance or a similar administrative purpose.
 - Vitamins, supplements, and proprietary preparations that are not prescribed for the treatment of a diagnosed illness or injury.
 - Treatment by a practitioner of a therapy that is not recognised as western medicine for the purposes of this Policy, unless the Rider Schedule states that a named alternative therapy is included.
 - Dental and optical treatment, which remain excluded under this Rider even where connected with a Visit that is otherwise covered.

- A Visit connected with a condition that was already being investigated or treated before the Rider Effective Date and that was not disclosed at the time this Rider was added.

A consultation conducted by real-time audio or video link with a Panel Doctor is treated as a Visit on the same terms as an in-person consultation, provided the Panel Doctor keeps a clinical record of the consultation in the same way as for an in-person Visit. A telehealth consultation with a Non-panel Doctor is reimbursed on the same reduced basis as an in-person Visit to a Non-panel Doctor. Where a telehealth consultation leads to a prescription, the medication itself is reimbursed on presentation of the dispensing receipt in the usual way, whether or not the pharmacy dispensing it is connected with the Panel Doctor who issued the prescription.

Allied health treatment prescribed in writing by a Physician, including speech therapy, occupational therapy and dietetic consultation for a diagnosed condition, is reimbursed under this Rider up to the number of sessions and the per-session limit shown in the Rider Schedule, counted separately from the physiotherapy sessions described in the Cover above and separately from the annual visit limit that applies to a Visit. Allied health treatment that is not prescribed in writing, or that continues beyond the number of sessions a Physician has certified as necessary, is not reimbursed under this paragraph, although it may still fall within the general annual visit limit if it is instead billed as a Visit to a Panel Doctor or a Non-panel Doctor.

Where both this Rider and the pre-admission or post-discharge outpatient benefit under Part B3 of the Policy Wording could apply to the same Visit, the Insured Person is not required to elect between them in advance; the Company applies the benefit under Part B3 first, since it is directly connected with a Confinement, and applies this Rider only to the extent, if any, that the Part B3 benefit or its own limit has not already met the Eligible Expenses of that Visit. This approach avoids the Insured Person needing to work out in advance which benefit is more favourable, while still preventing the same treatment from being paid twice.

The annual visit limit and the annual number of physiotherapy or allied health sessions shown in the Rider Schedule reset at the start of each Rider Year measured from the Rider Effective Date, which may fall on a different day of the year from the main Policy's own Anniversary Date where this Rider was added after the Policy Effective Date. Where the Rider Effective Date is later aligned with the main Policy's Anniversary Date, for example following a renewal on which the Rider Premium and the main Policy premium are consolidated onto a single date, the Rider Year in which the alignment occurs is shortened or lengthened as needed, and the annual limit for that single transitional Rider Year is adjusted in proportion to its length.

A.5 Critical Illness Lump-Sum Rider -- Policy Wording

This Rider pays a single lump sum on the first definite diagnosis of a Critical Illness that meets one of the descriptions given below, provided the Insured Person survives the Survival Period. Payment of this benefit is not connected with, and does not require, a Confinement, and it is paid in addition to any benefit payable under Part B of the Policy Wording for treatment of the same condition.

1. Critical Illness means one of the conditions listed below, diagnosed and evidenced in the manner described for that condition.
2. Diagnosis Date means the date on which a Specialist first makes a diagnosis that satisfies the description of a Critical Illness listed below, supported by the clinical and, where stated, the histological or radiological evidence described for that condition.
3. Specialist means a Physician who holds a recognised specialist qualification appropriate to the condition being diagnosed.

4. Survival Period means the period of 14 consecutive days beginning on the Diagnosis Date, during the whole of which the Insured Person must remain alive for the benefit to be payable.
5. Lump Sum Benefit means the single amount shown in the Rider Schedule that is payable once, in full, on the first valid claim under this Rider, after which this Rider terminates for that Insured Person.
1. Cancer. A malignant tumour characterised by the uncontrolled growth of malignant cells and invasion of tissue, confirmed by histological evidence, excluding a tumour that is histologically classified as pre-malignant, carcinoma in situ, or an early and localised skin cancer other than malignant melanoma with a Breslow depth greater than 1.5 millimetres.
2. Heart Attack. The death of a portion of heart muscle as a result of an inadequate blood supply, evidenced by a rise in cardiac enzymes or troponin together with new electrocardiographic changes consistent with the diagnosis.
3. Stroke. A cerebrovascular event causing infarction of brain tissue, haemorrhage, or embolisation from an extracranial source, with a resulting neurological deficit persisting for more than 24 hours and confirmed by imaging.
4. Kidney Failure. Chronic and irreversible failure of both kidneys, requiring the Insured Person to undergo regular dialysis or to receive a kidney transplant.
5. Major Organ Transplant. The Insured Person undergoing, as a recipient, a transplant of bone marrow or of a whole human heart, liver, lung, pancreas or kidney, or being placed on a recognised waiting list for such a transplant.
6. Coronary Artery Bypass Surgery. Surgery, proven necessary by angiographic evidence, to correct narrowing of one or more coronary arteries by bypass grafting, excluding angioplasty or a similar non-surgical procedure.
7. Paralysis. The total and permanent loss of use of two or more limbs as a result of injury or disease of the brain or spinal cord, persisting for a continuous period of at least three months.
8. Loss of Sight. The total and permanent loss of sight in both eyes, to the extent that visual acuity, even after correction, is assessed by a Specialist as beyond a recognised threshold of legal blindness.
9. Loss of Hearing. The total and permanent loss of hearing in both ears, with the hearing loss confirmed by audiometric testing to be beyond a recognised threshold.
10. Major Burns. Third-degree burns covering at least 20 percent of the body surface area, as certified by a Specialist.
11. Coma. A state of unconsciousness with no reaction to external stimuli, lasting continuously for at least 96 hours, requiring the use of life support systems, and resulting in a permanent neurological deficit.
12. Motor Neurone Disease. A definite diagnosis of a condition of progressive degeneration of the motor neurones, confirmed by a Specialist, with a persistent objective neurological deficit.
13. Multiple Sclerosis. A definite diagnosis by a Specialist, supported by imaging showing characteristic lesions, of at least two separate episodes of neurological deficit affecting different parts of the central nervous system.
14. Aplastic Anaemia. Chronic bone marrow failure resulting in anaemia, neutropenia and thrombocytopenia, confirmed by bone marrow biopsy, and requiring treatment with at least one of a blood product transfusion, a marrow stimulating agent, an immunosuppressive agent, or bone marrow transplantation.

15. Bacterial Meningitis. Inflammation of the membranes of the brain or spinal cord caused by a bacterial infection, confirmed by the isolation of the causative organism, resulting in a permanent neurological deficit persisting for at least three months.
16. Loss of Independent Existence. The total and permanent inability, following an Illness or Injury, to perform at least three of the six activities of daily living recognised for this purpose, namely washing, dressing, feeding, mobility, continence and transferring, where the inability has lasted continuously for at least six months and a Specialist certifies that it is permanent.

This Rider is subject to a waiting period of 90 days from the Rider Effective Date. A Critical Illness that is first diagnosed within that waiting period, or for which the earliest sign or symptom appeared before the Rider Effective Date, whether or not a diagnosis was made at the time, is not covered. Only one Lump Sum Benefit is payable to an Insured Person under this Rider in a lifetime, and this Rider terminates for that Insured Person once the Lump Sum Benefit has been paid; it does not revive if the Policy is later reinstated.

- A Critical Illness caused by a Pre-existing Condition that was not disclosed at the time this Rider was added.
- A Critical Illness diagnosed outside a properly licensed and equipped medical facility, or a diagnosis that is not supported by the clinical, histological or radiological evidence described for that condition.
- A condition that resembles, but does not meet, the full description of a Critical Illness listed above; a condition that is not listed above, however serious, is not covered under this Rider even though it may be covered under Part B of the Policy Wording.
- Any Critical Illness arising from a cause excluded under Part C of the Policy Wording, applied to this Rider in the same way as it applies to the main Policy.
- A second or subsequent occurrence of the same, or a different, Critical Illness after the Lump Sum Benefit has already been paid under this Rider.

Where an Insured Person is diagnosed with a condition that could reasonably be classified under more than one of the descriptions listed above, the Company treats it as a single Critical Illness for the purpose of the once-only Lump Sum Benefit, and pays under whichever description most closely matches the evidence presented; this does not entitle the Insured Person to more than one Lump Sum Benefit for what is, in substance, a single underlying diagnosis. Where a diagnosis is provisional and is later confirmed or revised by further investigation, the Diagnosis Date for the purpose of the waiting period and the Survival Period is the date of the diagnosis that is ultimately relied on to support the claim, not the date of an earlier provisional diagnosis that is not itself sufficient to satisfy the description of the Critical Illness concerned.

A claim under this Rider is assessed on the medical evidence available at the time the claim is made, including, where the Company considers it necessary, an examination by a Physician nominated by the Company; the cost of an examination requested by the Company is borne by the Company, while the cost of medical evidence the Insured Person obtains to support the claim is borne by the Insured Person unless the Company has asked for it to be obtained. A claim under this Rider is made using the same claim form as a claim under the main Policy, marked to show that it relates to the Critical Illness Lump-Sum Rider, together with the medical evidence described for the relevant Critical Illness above.

An applicant for this Rider is asked, as part of the application described in A.1 above, about the family history of a small number of hereditary conditions connected with the list of Critical Illnesses above; this question is separate from, and in addition to, the general duty of disclosure that applies to the applicant's own health. A family history disclosed at application does not by itself lead to the application being declined; it may instead lead to a request for further information, an adjustment to the Rider Premium, or,

in a small number of cases, a special exclusion recorded on the Rider Schedule for a specific hereditary condition.

A.6 Renewal, Variation and Termination of Riders

A Rider renews automatically together with the main Policy at each Anniversary Date for as long as the main Policy remains in force and the Rider Premium continues to be paid, subject to the Company's general right to review premiums and terms at renewal in accordance with Part D of the Policy Wording.

1. This Policy terminating for any reason terminates every Rider attached to it at the same time; a Rider cannot continue in force after the main Policy has ended.
2. The Policyholder may cancel a Rider on written notice taking effect from the next Anniversary Date, while keeping the main Policy and any other Rider in force; a Rider cannot be cancelled part-way through a Rider Year with a partial refund of the Rider Premium already paid.
3. The Company may withdraw a type of Rider from new sale, or amend its terms for future Rider Years, by giving notice before the relevant Anniversary Date; a Rider already attached continues on its existing terms until the Policyholder is separately offered, and accepts, replacement terms.
4. Non-payment of the Rider Premium when due is treated in the same way as non-payment of the premium for the main Policy under Part D of the Policy Wording, and may result in the Rider, or the whole Policy, lapsing.
5. The Critical Illness Lump-Sum Rider terminates for an Insured Person, separately from the rest of the Policy, once the Lump Sum Benefit has been paid for that Insured Person, as described in A.5 above.
6. A Dependant who ceases to meet the definition of Dependant in Part A of the Policy Wording, for example on reaching the age limit, ceases at the same time to be covered under any Rider, in the same way as under the main Policy.

Termination of a Rider does not affect a benefit that had already accrued before the termination date, provided that the treatment giving rise to the benefit was given, or the event giving rise to the benefit occurred, while the Rider was in force, and provided that the claim is made within the time limit that applies to the main Policy.

A notice given under this Supplement, whether by the Policyholder to cancel a Rider or by the Company to vary or withdraw a type of Rider, is given in the same manner and to the same address as a notice under the general provisions of Part D of the Policy Wording; it is not necessary to use a separate form for a notice relating only to a Rider, provided the notice identifies the Rider concerned and the Insured Person to whom it relates. A notice sent by post is treated as given on the date it would ordinarily be delivered in the usual course of post, and a notice given through the customer portal is treated as given when it is successfully submitted, in each case regardless of when it is actually read.

Where the main Policy lapses for non-payment of premium and is later reinstated in accordance with Part D of the Policy Wording, a Rider that was attached before the lapse is reinstated with it only if the Policyholder asks for the Rider to be reinstated and pays the outstanding Rider Premium together with the outstanding premium for the main Policy; the Company may require fresh medical evidence for the Rider on reinstatement even where none was required for the main Policy. A waiting period that had already been satisfied before the lapse is not required to run again on reinstatement, except for the Critical Illness Lump-Sum Rider, for which a fresh waiting period of 90 days runs from the date of reinstatement regardless of how long the Rider had previously been in force.

A Rider is not transferable to a different policy, and does not continue if the main Policy is replaced by a new policy, even where the replacement is with the Company and even where the Policyholder and the

Insured Persons remain the same; a Rider must be applied for afresh under the new policy, and is assessed as a new application in accordance with A.1 above. Where a group policy under which an Insured Person was covered is converted to an individual policy in accordance with the terms that apply to that conversion, a Rider that was attached under the group policy does not automatically carry over to the individual policy and must likewise be separately applied for.

A Rider described in this Supplement does not itself create a separate policy document; its terms are read together with, and are subject to, the general provisions of Parts D and E of the Policy Wording except where this Supplement expressly states otherwise.

Section 3. Claims Guide

This section explains how to prepare and submit a claim under the GuardianCare Hospital and Surgical Plan, what happens after the claim is received, and how the most common questions are answered. It should be read together with Part E (Claims Conditions) of the Policy Wording, which sets out the contractual requirements. Where this guide is more generous than Part E, Part E prevails.

3.1 Before you start

Collect the documents listed in the checklist printed at the top of the claim form. Original itemised bills and official receipts are required for every amount claimed; a credit card slip or a bank statement is not a receipt. Ask the Hospital for a discharge summary or a medical certificate that shows the diagnosis, the admission and discharge dates, and the treatment given. If the Insured Person was treated in a public hospital, the discharge slip issued at the ward reception is sufficient. Keep a copy of everything you send; originals are not returned unless requested at the time of submission.

3.2 Completing the claim form

The claim form has seven lettered sections, A to G, and must be completed by the Insured Person or, where the Insured Person is a child or is unable to sign, by the Policyholder or a close relative. Use block capitals and black ink if completing by hand. Every section that applies must be completed; a section that does not apply must be left blank. The three incident sub-sections under C are alternatives: complete the one that matches the Nature of Claim ticked, and leave the other two blank even where the information seems relevant. A form that has two incident sub-sections completed is returned for correction.

- Section A identifies the policy. Copy the policy and certificate numbers from the Policy Schedule or from the renewal notice; do not use the barcode reference printed on the form, which identifies the form and not the policy.
- Section B identifies the Insured Person. The name must match the identification document exactly, surname first. Tick one identification document type only and quote its number without spaces or hyphens other than those printed on the document.
- Section C describes the incident. The date of incident is the date the symptoms first appeared or the accident happened, not the admission date. For an illness, complete sub-section (1) with the diagnosing hospital and a short diagnosis; for a traffic accident, sub-section (2); for any other accident, sub-section (3).
- Section D lists the consultation milestones. Where a milestone did not occur, the cell is pre-printed Not Applicable and should be left as printed.
- Section E lists the expenses claimed by category and item. Enter amounts in Hong Kong dollars with cents. The total must equal the sum of the receipts enclosed; a total that does not reconcile delays assessment.
- Section F gives the payment instruction. Payment is made to the Policyholder unless a different payee is named and the relationship is ticked. For a bank transfer, quote the full account number as printed on the bank card or statement, including the bank's branch prefix.
- Section G is the declaration. It must be signed and dated in the presence of no witness; the Place is the city in which the form is signed.

3.3 How to submit

Claims may be submitted in any of three ways. By post, to the Claims Department at the address printed on the form. In person, at any branch of the Company during business hours, where the counter staff will

check the documents and issue a receipt. Online, through the customer portal, by uploading a scanned copy of the completed form and of each supporting document; originals must then be retained for six months and produced on request. The portal accepts PDF and image files of not more than 10 MB each and up to 20 files per claim.

3.4 What happens next

Receipt is acknowledged within 3 working days by letter or, for portal submissions, by e-mail. Straightforward claims are settled within 10 working days of receiving all required documents. Where the assessor needs further information, a request is sent within 5 working days setting out exactly what is needed and by when. A claim on which no response to such a request is received within 60 days is closed and may be reopened only by resubmission. On settlement, a statement of account is sent showing each item claimed, the amount allowed, the amount disallowed and the reason, and the net amount paid.

3.5 Frequently asked questions

Can I claim for a stay in a public hospital?

Yes. Where the Insured Person is confined in a general ward of a public hospital and pays only the standard ward charge, the public hospital daily cash benefit is paid instead of the reimbursement benefits. Submit the discharge slip showing the dates of stay; no itemised bill is required. Where a private ward or private patient service in a public hospital is used, the claim is assessed as a reimbursement claim in the usual way.

Do I need to tell you before I am admitted?

Notice within 30 days of admission is required under the Claims Conditions, but pre-authorisation is not. The Company recommends contacting the claims hotline before any planned admission so that the assessor can confirm the room type covered and the surgical limit that applies, which avoids surprises at settlement. For an emergency admission, notice may be given after discharge.

What if my employer's scheme has already paid part of the bill?

Submit the settlement advice from the employer's scheme together with photocopies of the receipts. The Company pays the balance of the eligible expenses up to the limits of the plan. Where the employer's scheme pays a fixed cash benefit rather than a reimbursement, that benefit is disregarded and the full eligible expenses are considered.

Why was my room and board reduced?

The room type fully covered depends on the Plan Level. Where a higher room type is occupied by choice, all confinement and surgical benefits for that stay are reduced in the same proportion as the room and board limit bears to the actual room charge. Where no room of the covered type was available, ask the Hospital for written confirmation and enclose it with the claim.

Can the benefit be paid to someone other than me?

Yes. Name the payee in Section F, tick the relationship, and sign the declaration. The payee's name on the bank account must match the name given. Payment to a third party who is not a close relative requires a separate letter of authority from the Policyholder.

How long do I have to submit?

Ninety days from the date of discharge or the last treatment. Late claims are considered where the delay is explained in writing and has not prejudiced assessment; a claim received more than one year after the treatment date is not considered.

What if the account number on file has changed?

Write the new account number in Section F of the claim form. The Company does not update payment details from a telephone call. Where the number is not yet known at the time of submission, write To be advised in Section F and provide the number in writing before settlement; the claim will be assessed but payment will be held until the number is received.

3.6 Specimen of a completed claim form

The two pages that follow show a specimen claim form completed for an imaginary illness claim, to illustrate how each section should be filled in. The particulars are fictitious and are printed for guidance only. Do not copy any value from the specimen into your own form.

SPECIMEN — FOR GUIDANCE ONLY — NOT A CLAIM

PACIFIC GUARDIAN LIFE ASSURANCE (H.K.) LIMITED
(Incorporated in Hong Kong with limited liability) SPECIMEN — FOR GUIDANCE ONLY
HOSPITALIZATION / ACCIDENT CLAIM FORM

HK0000000C00000
Please quote this reference in all correspondence

HOW TO SUBMIT THIS CLAIM

By post: Claims Department, 28/F Harbour Centre, 25 Harbour Road, Wan Chai, Hong Kong
In person: any branch of the Company during business hours
Online: upload the completed form and supporting documents through the customer portal

WHAT HAPPENS NEXT

We acknowledge receipt within 3 working days. Straightforward claims are settled within 10 working days of receiving all required documents. We will write to you if anything further is needed.

CLAIMS DOCUMENT CHECKLIST — tick each document enclosed

	Document		Document
☒	Completed and signed claim form	☒	Original itemised bills and official receipts
☒	Medical certificate or discharge summary	☒	Copy of the Insured Person's identification document
☐	Police report (traffic accident claims only)*	☐	Referral letter, where the treatment was referred

Part I To be completed by the Insured Person or Claimant

A. Policy Identification

Policy No. HK-MED-0000000	Certificate No. C-00000	Type of Claim ☐ Accident ☒ Hospitalization ☐ Outpatient
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B. Particulars of the Insured Person

Name of Insured (Surname first) Doe Jane (SPECIMEN)	Date of Birth (DD/MM/YYYY) 01/01/1980	Sex ☐ M ☒ F	Occupation / Position Occupation as on application
I.D. Card / Passport Type (tick one) ☒ National ID ☐ Passport ☐ Driver's License ☐ Other	I.D. Card / Passport No. SPECIMEN01	Contact No. 0000 0000	
Correspondence Address 1 Example Street, Central, Hong Kong	E-mail specimen@example.com		

C. Particulars of the Incident

*Illness — please complete section (1). Traffic accident — please complete section (2). Any other accident — please complete section (3). Leave the sections that do not apply blank.***

Date of Incident (DD/MM/YYYY) 01/01/2026	Time of Incident Not Applicable	Nature of Claim (tick one) ☐ Traffic Accident ☒ Ordinary Illness ☐ Common Accident
---	------------------------------------	---

(1) Illness Particulars

Diagnosis Hospital Canossa Hospital (Caritas)	Diagnosis Summary Acute gastroenteritis, treated conservatively
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(2) Traffic Accident Particulars

Place of Accident	Motor Vehicle Involved	Police Report No.^
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(3) Other Accident Particulars

Diagnosis Hospital	Part(s) of Body Injured	Severity
Diagnosis Summary		
Brief Description of the Incident Describe briefly how and when the illness began. Example: abdominal pain and vomiting for one day; admitted for observation.		

D. Consultation and Hospitalisation Particulars

	Date (DD/MM/YYYY)	Name and Address of Doctor / Hospital
(a) The doctor first consulted for this illness / injury	01/01/2026	Canossa Hospital (Caritas)
(b) Date of discharge, if the Insured Person was confined	02/01/2026	Canossa Hospital (Caritas)
(c) Level of hospital ward	Semi-private	Not Applicable
(d) Usual attending doctor, if different from the above	Not Applicable	Not Applicable

E. Expenses Claimed (all amounts in HK\$)

Category	Item	Amount HK\$
Outpatient	Examination and treatment fee	800.00
Outpatient	Medication fee	200.00
Outpatient	Medical service fee	0.00
Inpatient	Examination and treatment fee	6,000.00
Inpatient	Medication fee	900.00
Inpatient	Medical service fee	1,500.00
Inpatient	Sub-total of inpatient charges	8,400.00
	TOTAL AMOUNT CLAIMED#	9,400.00

F. Payment Instruction

Payee Name Doe Jane (SPECIMEN)	Relationship to the Insured (tick one) ☒ Self ☐ Spouse ☐ Parent ☐ Child ☐ Sibling ☐ Other	Payee Contact No. 0000 0000
Name of Bank Example Bank Limited	Bank Account No. 000-000000-000	

G. Declaration and Authorisation

I declare that the statements made in this form are true and complete to the best of my knowledge and that no material fact has been withheld. I authorise any hospital, physician or other person who has attended the Insured Person to release to the Company any information relating to this claim.

Signature of Claimant		Signature of Close Relative of Claimant (if applicable)	
Name of Claimant	I.D. Card No.	Date (DD/MM/YYYY)	Place
Doe Jane (SPECIMEN)	SPECIMEN01		Hong Kong

For Adviser's Use Only — Adviser Name	Adviser Code & Location	Received	Assessor

* Required only where the claim arises from a traffic accident.

** Completing a section that does not apply may delay assessment of the claim.

^ If the accident was reported to the police, quote the report number. Where no report was made, leave blank.

Claims must be submitted within 90 days of the incident. Late submission must be accompanied by a written explanation.

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END OF SPECIMEN — the particulars above are fictitious.

Section 4. Claim Form (completed)

The claim form for this claim follows on the next page. It has been completed by the Insured Person from the hospital documents and is the source of every value to be entered when the claim is lodged. Values in the specimen form in Section 3.6 are illustrations only and must not be used.

PACIFIC GUARDIAN LIFE ASSURANCE (H.K.) LIMITED

(Incorporated in Hong Kong with limited liability)

HOSPITALIZATION / ACCIDENT CLAIM FORM***HK3920554C71066***

Please quote this reference in all correspondence

HOW TO SUBMIT THIS CLAIM

By post: Claims Department, 28/F Harbour Centre, 25 Harbour Road, Wan Chai, Hong Kong
 In person: any branch of the Company during business hours
 Online: upload the completed form and supporting documents through the customer portal

WHAT HAPPENS NEXT

We acknowledge receipt within 3 working days. Straightforward claims are settled within 10 working days of receiving all required documents. We will write to you if anything further is needed.

CLAIMS DOCUMENT CHECKLIST — tick each document enclosed

	Document		Document
☒	Completed and signed claim form	☒	Original itemised bills and official receipts
☒	Medical certificate or discharge summary	☒	Copy of the Insured Person's identification document
☐	Police report (traffic accident claims only)*	☐	Referral letter, where the treatment was referred

Part I To be completed by the Insured Person or Claimant**A. Policy Identification**

Policy No. HK-MED-3920554	Certificate No. C-71066	Type of Claim ☐ Accident ☒ Hospitalization ☐ Outpatient
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B. Particulars of the Insured Person

Name of Insured (Surname first) Ng Chi Hang	Date of Birth (DD/MM/YYYY) 03/09/1975	Sex ☒ M ☐ F	Occupation / Position Secondary School Teacher
I.D. Card / Passport Type (tick one) ☐ National ID ☒ Passport ☐ Driver's License ☐ Other	I.D. Card / Passport No. P1140882	Contact No. 6113 7742	
Correspondence Address Room 704, Block C, Wing Cheung Building, 210 Nathan Road, Yau Ma Tei, Kowloon	E-mail ch.ng@mailbox.hk		

C. Particulars of the Incident

Illness — please complete section (1). Traffic accident — please complete section (2). Any other accident — please complete section (3). Leave the sections that do not apply blank.**

Date of Incident (DD/MM/YYYY) 22/02/2026	Time of Incident Not Applicable	Nature of Claim (tick one) ☐ Traffic Accident ☒ Ordinary Illness ☐ Common Accident
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(1) Illness Particulars

Diagnosis Hospital Queen Elizabeth Hospital	Diagnosis Summary Acute appendicitis, treated by laparoscopic appendectomy
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(2) Traffic Accident Particulars

Place of Accident	Motor Vehicle Involved	Police Report No.^
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(3) Other Accident Particulars

Diagnosis Hospital	Part(s) of Body Injured	Severity
Diagnosis Summary		
Brief Description of the Incident Persistent right lower abdominal pain with fever over two days; admitted the same evening for laparoscopic appendectomy.		

D. Consultation and Hospitalisation Particulars

	Date (DD/MM/YYYY)	Name and Address of Doctor / Hospital
(a) The doctor first consulted for this illness / injury	22/02/2026	Queen Elizabeth Hospital
(b) Date of discharge, if the Insured Person was confined	25/02/2026	Queen Elizabeth Hospital
(c) Level of hospital ward	Semi-private	Not Applicable
(d) Usual attending doctor, if different from the above	Not Applicable	Not Applicable

E. Expenses Claimed (all amounts in HK\$)

Category	Item	Amount HK\$
Outpatient	Examination and treatment fee	2,240.00
Outpatient	Medication fee	310.00
Outpatient	Medical service fee	0.00
Inpatient	Examination and treatment fee	18,600.00
Inpatient	Medication fee	3,450.00
Inpatient	Medical service fee	7,900.00
Inpatient	Sub-total of inpatient charges	29,950.00
	TOTAL AMOUNT CLAIMED#	32,500.00

F. Payment Instruction

Payee Name Ng Chi Hang		Relationship to the Insured (tick one) ☒ Self ☐ Spouse ☐ Parent ☐ Child ☐ Sibling ☐ Other	Payee Contact No. 6113 7742
Name of Bank Bank of China (Hong Kong)		Bank Account No. 012-445097-201	

G. Declaration and Authorisation

I declare that the statements made in this form are true and complete to the best of my knowledge and that no material fact has been withheld. I authorise any hospital, physician or other person who has attended the Insured Person to release to the Company any information relating to this claim.

Signature of Claimant		Signature of Close Relative of Claimant (if applicable)	
Name of Claimant	I.D. Card No.	Date (DD/MM/YYYY)	Place
Ng Chi Hang	P1140882		Hong Kong

For Adviser's Use Only — Adviser Name	Adviser Code & Location	Received	Assessor

* Required only where the claim arises from a traffic accident.

** Completing a section that does not apply may delay assessment of the claim.

^ If the accident was reported to the police, quote the report number. Where no report was made, leave blank.

Claims must be submitted within 90 days of the incident. Late submission must be accompanied by a written explanation.

Section 5. Supporting Documents and Enclosures

The supporting documents for this claim are enclosed as separate originals and are not reproduced in this bundle. The enclosure list below records what was submitted with the claim form in Section 4. The blank medical certificate that follows is the Company's standard form and is included so that the attending Physician may complete it where the Hospital's own discharge summary does not state the diagnosis.

5.1 Enclosure list

No.	Document	Pages	Original / Copy
1	Completed and signed claim form (Section 4 of this bundle)	2	Original
2	Hospital discharge summary signed by the attending physician	2	Original
3	Itemised hospital bill covering the period of confinement	3	Original
4	Official receipts for the inpatient charges	3	Original
5	Official receipts for the pre-admission outpatient consultation and medication	2	Original
6	Copy of the Insured Person's passport data page	1	Copy
7	Copy of the payee's bank card showing the account number	1	Copy

Where an original is required and a copy is supplied, the claim is held until the original is received.

5.2 Medical certificate (blank Company form)

To be completed by the attending Physician. Please print clearly. The Physician's signature and the Hospital's chop are required.

Item	Entry
('Name of patient', '')	
('Identification document no.', '')	
('Date symptoms first appeared', '')	
('Date of first consultation', '')	
('Date of admission', '')	
('Date of discharge', '')	
('Final diagnosis', '')	
('ICD-10 code', '')	
('Treatment given, including any operation performed', '')	
('Was the condition related to any pre-existing condition? If yes, give details', '')	
('Was the admission medically necessary? If so, why could the treatment not be given as an outpatient?', '')	
('Type of ward occupied', '')	
('Name of attending physician', '')	
('Qualifications and registration number', '')	
('Hospital name and chop', '')	
('Signature and date', '')	

Section 6. Correspondence Templates

The letters in this section are the standard texts used by the Claims Department at each stage of a claim. They are reproduced so that a claimant knows what to expect and can recognise a genuine communication from the Company. Every letter sent will carry the claim reference, the date, and the name and direct line of the assessor handling the claim.

6.1 Acknowledgement of receipt

Reference: [as shown on the acknowledgement] Date: [date of issue]

Dear Policyholder,

Thank you for your claim under the GuardianCare Hospital and Surgical Plan, which we received on the date shown above. It has been passed to the assessor named below, who will review the documents within five working days.

If everything we need has been provided, we expect to settle the claim within ten working days of the date of this letter. If we need anything further, we will write to you separately setting out what is required. You do not need to contact us in the meantime.

Please quote the claim reference in any correspondence. Original documents will be retained on file for six months and then securely destroyed unless you ask us to return them.

Yours faithfully,

Lee Ha-eun
Claims Assessor, Hospital Claims Team
Direct line 2830 4471

6.2 Request for further information

Reference: [as shown on the acknowledgement] Date: [date of issue]

Dear Policyholder,

We have reviewed the claim referenced above and need the following before we can complete our assessment:

1. The original itemised hospital bill. The document received is a summary statement that does not show the individual charges.
2. A medical certificate or discharge summary stating the final diagnosis and the dates of admission and discharge, signed by the attending physician.
3. Confirmation of the room type occupied, where the bill shows a room charge above the level covered by your plan.

Please send the documents within 60 days of the date of this letter. If we do not hear from you by then, the claim will be closed and may be reopened only by resubmission of the complete claim.

Yours faithfully,

Thomas Reid
Senior Claims Assessor, Hospital Claims Team
Direct line 2830 4466

6.3 Settlement advice

Reference: [as shown on the acknowledgement] Date: [date of issue]

Dear Policyholder,

We are pleased to advise that the claim referenced above has been assessed and settled. The enclosed statement of account shows each item claimed, the amount allowed under your plan, any amount not allowed and the reason, and the net amount paid.

Payment has been made by bank transfer to the account nominated in Section F of your claim form and should reach the account within three working days. Where payment was requested by cheque, the cheque is enclosed.

If you disagree with any part of the assessment, please write to us within 30 days stating the item concerned and the reason. Section 8 of your policy bundle explains how to make a complaint and how to refer a dispute to the Insurance Complaints Bureau if it cannot be resolved with us.

Yours faithfully,

Lee Ha-eun
Claims Assessor, Hospital Claims Team
Direct line 2830 4471

6.4 Notice of decline

Reference: [as shown on the acknowledgement] Date: [date of issue]

Dear Policyholder,

We have completed our assessment of the claim referenced above and regret that we are unable to admit it. The reason is stated below with reference to the relevant provision of the Policy Wording.

Reason: the condition treated first manifested itself before the Policy Effective Date and was not disclosed in the application. Under Part C, Exclusion 1, a Pre-existing Condition that was not disclosed is excluded from cover. The medical records obtained with your authority show consultations for the same condition in the twelve months before the application.

If you have information that you believe we have not taken into account, please send it to us within 30 days and we will review the decision. You may also refer the matter to the Insurance Complaints Bureau as described in Section 8 of your policy bundle.

Yours faithfully,

Thomas Reid
Senior Claims Assessor, Hospital Claims Team
Direct line 2830 4466

Section 7. Schedules, Tables and Directories

7.1 Schedule of Benefits (per Insured Person, HK\$)

Benefit	Standard	Superior	Premier
Annual benefit limit	400,000	800,000	1,600,000
Lifetime benefit limit	2,000,000	4,000,000	8,000,000
Room and board, per day (max 180 days)	900	1,800	3,600
Intensive care, per day (max 30 days)	3,000	6,000	12,000
Attending physician's visit, per day (max 180 days)	700	1,400	2,800
Specialist consultation, per confinement	3,500	7,000	14,000
Hospital services, per confinement	20,000	40,000	80,000
Private nursing, per day (max 30 days)	600	1,200	2,400
Surgeon's fee, per procedure (see Surgical Schedule)	up to 30,000	up to 60,000	up to 120,000
Anaesthetist's fee	35% of surgeon's fee	35% of surgeon's fee	35% of surgeon's fee
Operating theatre charge	35% of surgeon's fee	35% of surgeon's fee	35% of surgeon's fee
Pre-admission outpatient, per confinement	1,500 (30 days)	3,000 (30 days)	6,000 (60 days)
Post-discharge outpatient, per confinement	2,000 (60 days)	4,000 (90 days)	8,000 (90 days)
Home nursing, per day (max 30 days)	500	1,000	2,000
Public hospital daily cash, per day (max 90 days)	500	800	1,200
Accidental emergency outpatient, per accident	2,000	4,000	8,000
Emergency dental, per accident	3,000	6,000	12,000
Ambulance, per confinement	1,000	2,000	4,000
Companion bed, per day (max 30 days, child under 12)	300	600	1,200
Compassionate death benefit	10,000	20,000	40,000
Overseas emergency treatment	Not covered	90 days per trip	180 days per trip

7.2 Surgical Schedule (extract — surgeon's fee limits, Superior level, HK\$)

Procedure	Category	Limit
Appendectomy, open or laparoscopic	Intermediate	24,000
Cholecystectomy, laparoscopic	Major	36,000
Inguinal hernia repair, unilateral	Intermediate	20,000
Haemorrhoidectomy	Minor	12,000
Tonsillectomy	Minor	10,000
Septoplasty	Intermediate	18,000
Arthroscopy of knee, diagnostic	Minor	14,000
Arthroscopic meniscectomy	Intermediate	24,000
Anterior cruciate ligament reconstruction	Major	48,000
Open reduction and internal fixation, forearm	Major	40,000
Open reduction and internal fixation, ankle	Major	42,000
Carpal tunnel release	Minor	9,000
Excision of ganglion	Minor	7,000
Cataract extraction with intraocular lens, one eye	Intermediate	22,000
Colonoscopy with polypectomy	Minor	12,000
Upper gastrointestinal endoscopy, diagnostic	Minor	6,000
Chest drain insertion, closed	Minor	8,000
Video-assisted thoracoscopic surgery, wedge resection	Major	54,000
Thyroidectomy, total	Major	48,000
Mastectomy, simple	Major	40,000
Hysterectomy, abdominal	Major	44,000
Transurethral resection of prostate	Major	38,000
Coronary angiography, diagnostic	Intermediate	16,000
Percutaneous coronary intervention, single vessel	Complex	60,000
Coronary artery bypass graft	Complex	60,000

Craniotomy for tumour	Complex	60,000
Spinal decompression, single level	Major	50,000
Total hip replacement	Complex	60,000
Total knee replacement	Complex	60,000
Skin lesion excision, with suture	Minor	4,000

The full Surgical Schedule lists over 900 procedures and is available from the customer portal. Category limits at the Standard and Premier levels are one half and twice the Superior amounts respectively.

7.3 Annual Premium Table (HK\$, per Insured Person, effective 1 January 2026)

Premiums are determined by the attained age of the Insured Person at each Anniversary Date and by the Plan Level. The rates below apply to non-smokers ordinarily resident in Hong Kong; a loading of 20 percent applies to smokers. Rates are not guaranteed and may be revised on 30 days' notice.

Attained age	Standard	Superior	Premier
15 days – 4	3,120	5,460	9,840
5 – 9	2,640	4,620	8,320
10 – 14	2,410	4,220	7,600
15 – 19	2,560	4,480	8,060
20 – 24	2,890	5,060	9,110
25 – 29	3,240	5,670	10,210
30 – 34	3,680	6,440	11,590
35 – 39	4,270	7,470	13,450
40 – 44	5,110	8,940	16,090
45 – 49	6,230	10,900	19,620
50 – 54	7,760	13,580	24,440
55 – 59	9,840	17,220	31,000
60 – 64	12,650	22,140	39,850
65 – 69	16,300	28,530	51,350
70 – 74	20,900	36,580	65,840
75 – 79	26,700	46,730	84,110
80 – 84 (Premier only)	—	—	106,900

7.4 Deductible discount (Premier level only)

Elected deductible per policy year	Premium discount
Nil	0%
HK\$ 20,000	22%
HK\$ 50,000	38%
HK\$ 100,000	51%

7.5 Payment mode factors

Payment mode	Factor applied to annual premium	Effective annual cost
Annual	1.000	100.0%
Semi-annual	0.520 per instalment	104.0%
Quarterly	0.265 per instalment	106.0%
Monthly	0.090 per instalment	108.0%

7.6 Network Hospital Directory

Confinement at a network hospital allows the Hospital to bill the Company directly for the covered portion of the charges, on production of the membership card and a pre-authorisation reference. Confinement elsewhere is reimbursed in the usual way. The directory is reviewed quarterly; the current list is available on the customer portal.

Hospital	District	Direct billing	Hotline
Harbourview Adventist Hospital	Happy Valley	Yes	2574 6211
Matilda Peak Hospital	The Peak	Yes	2849 0111
Canossa Hospital (Caritas)	Mid-Levels	Yes	2522 2181
St. Paul's Hospital	Causeway Bay	Yes	2890 6008
Gleneagles Hong Kong Hospital	Wong Chuk Hang	Yes	3153 9000
Hong Kong Sanatorium and Hospital	Happy Valley	Yes	2572 0211
St. Teresa's Hospital	Kowloon City	Yes	2200 3434
Evangel Hospital	Kowloon City	Yes	2711 5221
Precious Blood Hospital (Caritas)	Sham Shui Po	Yes	2386 4471
Union Hospital	Sha Tin	Yes	2608 3388
Baptist Hospital	Kowloon Tong	Yes	2339 8888
Hong Kong Adventist Hospital – Tsuen Wan	Tsuen Wan	Yes	2275 6688
CUHK Medical Centre	Sha Tin	Yes	3946 6888
St. Clement's Hospital	Tsim Sha Tsui	Yes	2721 0033
Westbourne International Hospital	Kwun Tong	Yes	2389 7100
Victoria Harbour Hospital	North Point	Yes	2566 3020
Bayside Medical Centre	Tuen Mun	No	2459 8810
Kingsley Hospital	Yuen Long	No	2478 1122
Greenfield Day Surgery Centre	Central	Yes	2521 4460
Meridian Day Surgery Centre	Mong Kok	Yes	2395 7788
Queen Mary Hospital (public)	Pok Fu Lam	Not applicable	2255 3838
Queen Elizabeth Hospital (public)	Yau Ma Tei	Not applicable	3506 8888
Prince of Wales Hospital (public)	Sha Tin	Not applicable	3505 2211
Princess Margaret Hospital (public)	Kwai Chung	Not applicable	2990 1111
Tuen Mun Hospital (public)	Tuen Mun	Not applicable	2468 5111
United Christian Hospital (public)	Kwun Tong	Not applicable	3949 4000
Pamela Youde Nethersole Eastern Hospital (public)	Chai Wan	Not applicable	2595 6111
Ruttonjee Hospital (public)	Wan Chai	Not applicable	2291 2000
Kwong Wah Hospital (public)	Yau Ma Tei	Not applicable	3517 5000
Caritas Medical Centre (public)	Sham Shui Po	Not applicable	3408 7911
Yan Chai Hospital (public)	Tsuen Wan	Not applicable	2417 8383
North District Hospital (public)	Sheung Shui	Not applicable	2683 8888
Alice Ho Miu Ling Nethersole Hospital (public)	Tai Po	Not applicable	2689 2000
Tseung Kwan O Hospital (public)	Tseung Kwan O	Not applicable	2208 0111
Pok Oi Hospital (public)	Yuen Long	Not applicable	2486 8000
Tin Shui Wai Hospital (public)	Tin Shui Wai	Not applicable	3513 5000

7.7 Panel clinics for pre-admission and post-discharge outpatient care (extract)

Clinic	Speciality	District	Telephone
Central Health Medical Practice	General practice	Central	2824 0822
Harbour Orthopaedic Centre	Orthopaedics	Central	2537 6363
Wanchai Cardiac Clinic	Cardiology	Wan Chai	2892 0170
Island Respiratory Clinic	Respiratory medicine	Causeway Bay	2576 8181
Kowloon Gastro Clinic	Gastroenterology	Tsim Sha Tsui	2367 4141
Nathan Road Family Clinic	General practice	Jordan	2735 5060
Mong Kok Surgical Centre	General surgery	Mong Kok	2381 2299

Sha Tin Physiotherapy Centre	Physiotherapy	Sha Tin	2691 0533
Tsuen Wan Medical Practice	General practice	Tsuen Wan	2411 7272
Tai Po Family Health	General practice	Tai Po	2665 3311
Tuen Mun Diagnostic Imaging	Radiology	Tuen Mun	2450 9090
Kwun Tong Eye Centre	Ophthalmology	Kwun Tong	2342 6655
Quarry Bay ENT Clinic	Otorhinolaryngology	Quarry Bay	2562 2020
Aberdeen Dental Surgery	Dentistry (accidental only)	Aberdeen	2555 1188
Sai Kung Medical Centre	General practice	Sai Kung	2792 8383

7.8 Glossary of terms used in claims correspondence

The following terms appear in statements of account and in letters from the Claims Department. They are explained here in plain language; where a term is defined in the Policy Wording, the Policy Wording definition governs.

Term	Meaning
Allowed amount	The part of an item claimed that falls within the benefit and its limit and is therefore paid.
Assessor	The member of the Claims Department responsible for reviewing a claim and deciding what is payable.
Benefit limit	The maximum the plan pays for a particular benefit, per day, per confinement or per policy year, as shown in the Schedule of Benefits.
Co-payment	A share of eligible expenses that the insured person bears. This plan has no co-payment; it may have a deductible if elected.
Day case	A surgical procedure where the patient is admitted and discharged on the same day and no room charge is made.
Direct billing	An arrangement under which a network hospital bills the Company directly for the covered portion of the charges.
Disallowed amount	The part of an item claimed that is not paid, with the reason stated on the statement of account.
Discharge summary	The document issued by the hospital at discharge that states the diagnosis, the dates of stay and the treatment given.
Endorsement	A written amendment to the policy issued by the Company, for example to add a dependant or to exclude a condition.
Excess	Another word for deductible: the amount the policyholder bears in each policy year before benefit is paid.
Itemised bill	A hospital bill that lists each charge separately with its date, description and amount, as opposed to a summary statement.
Loading	An addition to the standard premium applied to reflect a higher risk, for example for smokers or for a disclosed condition.
Medically necessary	Treatment that is appropriate to the diagnosis, consistent with good medical practice, and not primarily for convenience.
Network hospital	A hospital listed in the directory in this section, at which direct billing is available.
Pre-authorisation	Confirmation given by the Company before a planned admission of the room type and surgical limit that will apply.
Pre-existing condition	A condition for which the insured person had symptoms, advice or treatment before the policy started.
Reasonable and customary	The level of charge normally made in the locality for the same treatment by providers of similar standing.
Room type adjustment	The proportional reduction applied when a higher room type than that covered is occupied by choice.
Statement of account	The document sent on settlement that shows what was claimed, what was allowed, what was disallowed and why, and what was paid.
Waiting period	The first 30 days of cover, during which illness is not covered. Accidents are covered from the first day.

7.9 Contact Directory

Function	Contact	Hours
Claims hotline (hospital claims)	2830 4400	Monday to Friday 9:00–18:00, Saturday 9:00–13:00
Pre-authorisation line	2830 4420	Monday to Friday 9:00–18:00
Customer service (policy administration)	2830 4000	Monday to Friday 9:00–18:00
Customer portal support	2830 4050	Daily 8:00–22:00
Customer Relations Unit (complaints)	2830 4900	Monday to Friday 9:00–18:00
Data Protection Officer	In writing to the principal office	—
Principal office	28/F Harbour Centre, 25 Harbour Road, Wan Chai, Hong Kong	Monday to Friday 9:00–18:00
Insurance Complaints Bureau	29/F Sunshine Plaza, 353 Lockhart Road, Wan Chai; 2520 2728	Monday to Friday 9:00–17:30
Emergency assistance (overseas, Superior and Premier)	+852 2830 4499 (24 hours)	24 hours

7.10 Branch offices

Branch	Address	Telephone
Central	12/F Prosperity Tower, 39 Queen's Road Central	2830 4101
Causeway Bay	5/F Times Square Tower One, 1 Matheson Street	2830 4102
Tsim Sha Tsui	8/F Silvercord Tower 2, 30 Canton Road	2830 4103
Mong Kok	11/F Grand Century Place Tower 2, 193 Prince Edward Road West	2830 4104
Kwun Tong	6/F Millennium City 5, 418 Kwun Tong Road	2830 4105
Sha Tin	3/F New Town Tower, 10 Pak Hok Ting Street	2830 4106
Tsuen Wan	9/F Nan Fung Centre, 264 Castle Peak Road	2830 4107
Tuen Mun	4/F Tuen Mun Central Square, 22 Hoi Wing Road	2830 4108

Section 8. Complaints Procedure

The Company aims to handle every claim fairly and promptly. If you are dissatisfied with any aspect of the service you have received, or with a decision on a claim, the following procedure applies.

8.1 Step one — the team that handled your claim

Contact the assessor named on the correspondence, by telephone on the direct line given or in writing. Most concerns are resolved at this stage by explaining the basis of the decision or by correcting an error. The assessor will respond within five working days.

8.2 Step two — the Customer Relations Unit

If the matter is not resolved to your satisfaction, write to the Customer Relations Unit at the Company's principal office, marking the envelope or the e-mail subject Complaint. State the claim reference, what happened, what you believe should have happened, and what outcome you are seeking. The Unit acknowledges every complaint within three working days and gives a full written response within 30 days. Where the investigation takes longer, you will be told why and when a response can be expected.

8.3 Step three — the Insurance Complaints Bureau

If you remain dissatisfied after receiving the Company's final response, or if 30 days have passed without a final response, you may refer the matter to the Insurance Complaints Bureau, an independent body that resolves disputes between insurers and their personal-lines customers without charge. The Bureau considers complaints about the outcome of a claim where the amount in dispute does not exceed its published limit, and its decision is binding on the Company but not on the complainant. The referral must be made within six months of the Company's final response. The Bureau's contact details are given in the contact directory in Section 7 of this bundle.

8.4 What the procedure does not cover

The procedure does not apply to complaints about the premium rate, the terms of the plan, or underwriting decisions, which are matters of commercial judgement for the Company. It also does not apply to matters that are already the subject of legal proceedings. Nothing in this procedure affects your legal rights.

Section 9. Personal Information Collection Statement

This statement is made in accordance with the Personal Data (Privacy) Ordinance and explains how the Company collects, uses and shares the personal data of policyholders, insured persons, claimants and payees.

9.1 Purposes of collection

Personal data provided in an application, a claim form, or in the course of correspondence is collected for the purposes of processing the application and any subsequent claim; administering, servicing and renewing the policy; verifying identity and detecting and preventing fraud; conducting underwriting, actuarial, statistical and research studies; complying with legal and regulatory requirements in and outside Hong Kong; and, subject to consent, informing the policyholder of other products and services offered by the Company. Provision of the data requested on the claim form is voluntary, but the Company may be unable to assess a claim if the data is not provided.

9.2 Transfer of data

Personal data may be transferred to the Company's reinsurers, claims investigators, medical examiners, legal advisers and other service providers, in Hong Kong or elsewhere, for the purposes stated above; to any person to whom the Company is under a legal or regulatory duty to disclose; to the Insurance Fraud Prevention Claims Database maintained by the Hong Kong Federation of Insurers, where the data may be accessed by other insurers for the purpose of detecting fraud; and to any actual or proposed assignee of the Company's business. The Company requires every recipient to keep the data confidential and to use it only for the purpose for which it was transferred.

9.3 Sensitive data

Medical information supplied in support of a claim is used only for the assessment of that claim and of any future claim under the same policy, and for the detection of fraud. It is stored separately from general policy records, is accessible only to claims and medical staff, and is not used for marketing.

9.4 Retention

Claim records are retained for seven years after the policy terminates, or longer where required by law or where a dispute is pending. Original supporting documents are retained for six months after settlement and then securely destroyed unless their return was requested at submission.

9.5 Access and correction

A data subject may request access to, or correction of, personal data held by the Company by writing to the Data Protection Officer at the Company's principal office. The Company may charge a reasonable fee for complying with an access request and will respond within 40 days. A data subject may also withdraw consent to the use of data for marketing at any time, without charge, by writing to the same address.

9.6 Enquiries

Enquiries about this statement or about the Company's handling of personal data should be addressed to the Data Protection Officer, whose contact details appear in the contact directory in Section 7.

Section 10. Document Control

Version	Date	Change	Approved by
1.0	01/01/2019	First issue of the GuardianCare bundle format	Product Committee
1.1	01/07/2019	Surgical Schedule extract extended to 30 procedures	Chief Medical Officer
2.0	01/01/2021	Premier level introduced; overseas emergency benefit added	Product Committee
2.1	01/07/2022	Claims Guide FAQ added; specimen form added	Head of Claims
2.2	01/01/2024	Personal Information Collection Statement revised	Data Protection Officer
3.0	01/01/2026	Claim form L-CL05-JAN26 adopted; premium table 2026; directory refreshed	Product Committee

This bundle is issued to a policyholder on request when a claim is lodged, so that the claim form, the terms under which it will be assessed, and the supporting material are kept together. The bundle is not itself a policy document and confers no rights. The claim form in Section 4 is the only part of this bundle that is completed by the claimant; all other sections are for reference.

End of bundle.

Annex A -- Pre-authorisation and Case Management

This Annex explains the process the Company uses to review a planned course of treatment before it is given, so that an Insured Person and the treating Physician know, so far as this is possible in advance, whether the treatment appears to fall within the cover provided by this Policy. It supplements, and should be read together with, the Claims Guide elsewhere in this bundle, which describes how a claim is submitted once treatment has been completed. The process described in this Annex does not replace the need to submit a claim; it is a separate step that, where it is used, can make the claim itself more straightforward to assess.

The arrangements described in this Annex apply to the main Policy and to a Rider attached to it, where the Rider's own terms call for pre-authorisation; a reference in this Annex to a benefit, a limit or an exclusion is a reference to whichever of the main Policy or a Rider the treatment in question falls under. Nothing in this Annex changes the benefits, the limits or the exclusions described elsewhere in this bundle: pre-authorisation and the case management arrangements that go with it affect only the timing and the administration of a claim, not its substance.

Annex A.1 When Pre-authorisation Is Required

Pre-authorisation means the Company's written confirmation, given before treatment is provided, that a described course of treatment appears, on the clinical information available at the time, to fall within the cover and the limits of this Policy. It is not itself a decision on a claim: the Company's final assessment is made after treatment has been given and the supporting documents required by the Claims Guide have been received.

1. A planned Confinement, other than one arising from an emergency, including a Confinement for Surgery, for investigation of a condition that a Physician expects to require several days of inpatient care, or for a Delivery under the Maternity Rider where that Rider is attached.
2. A Day Case Procedure that a Physician has recommended and scheduled in advance.
3. Treatment that a Physician expects will require the use of an Intensive Care Unit for a defined period.
4. A course of treatment expected to continue, or to be reviewed, over more than one Confinement, so that the case management team can follow the course of treatment as a whole rather than assessing each Confinement separately.
5. A claim that is expected to be made under the Critical Illness Lump-Sum Rider, where the Insured Person or the treating Physician wishes the supporting clinical evidence to be reviewed in advance of a formal claim being lodged.
6. Continuation of overseas emergency treatment under the relevant benefit in Part B4 of the Policy Wording, once the Insured Person's condition has stabilised sufficiently for a plan for further treatment to be discussed with a case manager.

Pre-authorisation is not required for an emergency admission: in that situation the priority is to obtain appropriate treatment without delay, and the Company only asks to be notified, as described in Annex A.2 below, within a reasonable time after admission. Pre-authorisation is also not required for the outpatient benefits described in Part B3 of the Policy Wording, in the Outpatient Top-Up Rider, or in the Dental Care Rider, where the amounts involved and the nature of the treatment do not call for advance review.

Where an Insured Person is admitted while travelling and the usual channels described in Annex A.2 below are not readily available, for example because of the time difference or a language barrier at the point of admission, the priority remains obtaining appropriate treatment; a family member or the

admitting facility may notify the Company once this becomes practicable, and the case manager who takes on the file will treat the notification as timely provided it is made as soon as the circumstances reasonably allow. This does not extend the separate time limit that applies to lodging the claim itself once treatment has been completed.

Pre-authorisation is an indication given in advance, based on the information available at the time. It is not a guarantee that a benefit will be paid, and it does not replace the assessment carried out once a claim, with its supporting documents, has been submitted.

Where pre-authorisation was available for a planned course of treatment but was not requested, this does not by itself prevent a benefit from being paid. It does mean that the clinical justification for the treatment, and its consistency with the terms of this Policy, is reviewed for the first time only once the claim is submitted, which can take longer than a claim for treatment that was already reviewed in advance.

An Insured Person or a treating Physician who is unsure whether a particular course of treatment falls within the list above is encouraged to ask, rather than to guess; a short enquiry to the case management team, describing the treatment in general terms, is answered without the formality of a full request and does not commit the Insured Person to proceeding with a request if the answer is that pre-authorisation is not required. A borderline case, for example a day case procedure that may extend into an overnight stay depending on how the Insured Person responds to treatment, is treated as requiring pre-authorisation, since the more cautious course avoids an unnecessary retrospective review later.

Where more than one Rider or benefit could apply to the same course of treatment, for example where a planned Delivery under the Maternity Rider is also expected to involve a stay in an Intensive Care Unit for the Newborn Child, a single request for pre-authorisation covering the whole course of treatment is preferred to separate requests for each element, so that the case manager assigned to the file can coordinate the assessment of all the benefits that may be relevant.

A course of treatment for a chronic condition that is managed in stages, where each stage is separately reviewed by the treating Physician before the next is recommended, is not treated as a single planned Confinement requiring one pre-authorisation covering every future stage; each stage that itself meets the description in the list above is instead the subject of its own request, made once the treating Physician has recommended it, so that the assessment reflects the Insured Person's condition at the time each stage is actually needed rather than a plan drawn up before the earlier stages have been completed.

Annex A.2 How to Request Pre-authorisation and Turnaround Times

A request for pre-authorisation may be made by the Insured Person, by a family member on the Insured Person's behalf, or by the admission office of the Hospital where the Insured Person has given consent for it to do so. A request is made using the channels described for this purpose elsewhere in the Claims Guide, and is a separate step from lodging the claim form itself once treatment is complete.

1. The request is submitted together with the treating Physician's initial clinical information, including the provisional diagnosis, the proposed treatment, and the expected length of stay or the expected duration of the course of treatment.
2. A case manager logs the request and reviews the clinical information against the terms of this Policy, including the relevant Benefit Limit, any applicable waiting period, and any exclusion that may be relevant.
3. Where the information provided is not sufficient for a decision to be made, the case manager writes to the person who submitted the request, and, where appropriate, to the treating Physician directly, identifying what further information is needed.

4. A decision is issued in writing, confirming whether the described treatment is pre-authorised, is pre-authorised subject to a stated condition or limit, is deferred pending further information, or cannot be pre-authorised on the information available.
5. A pre-authorisation, once given, remains valid for the period stated in the decision letter, ordinarily 30 days from the date of issue, within which the treatment described must begin for the pre-authorisation to apply without a fresh request.

Category	When to notify	Target turnaround
Planned Confinement or Surgery	At least 5 business days before the planned admission date	Decision issued within 3 business days of complete clinical information
Planned Day Case Procedure	At least 3 business days before the procedure	Decision issued within 2 business days
Urgent but non-emergency admission	As soon as the need is known, ordinarily at least 24 hours before admission	Decision issued within 24 hours of complete clinical information
Emergency admission	Notification only, within 48 hours after admission	Acknowledgement within 24 hours; a case manager assigned within 2 business days
Extension of an approved period	Before the current validity period expires	Decision issued within 2 business days

Where a planned admission is delayed beyond the validity period of a pre-authorisation, a short extension may be requested by writing to the case manager who issued the original decision, quoting the reference given in that decision. The case manager will confirm whether the original clinical information remains current or whether an updated assessment from the treating Physician is needed before the extension is granted.

A request for pre-authorisation is made in writing, using the channels described in the Claims Guide, and should identify the Insured Person, the treating Physician, the Hospital or day surgery centre where treatment is expected to take place, and the nature of the treatment in enough clinical detail for a case manager to assess it against this Policy. A request that does not identify these matters is returned to the person who submitted it with a note of what is missing, and the clock for the target turnaround described below does not begin to run until the missing information has been provided.

The clinical information that most often allows a request to be decided at the first review includes a short written summary from the treating Physician of the working diagnosis and the treatment proposed, any relevant laboratory or imaging report already available, and, where the request follows an earlier outpatient consultation, a copy of the referral letter recommending admission or surgery. A request supported only by an appointment confirmation, without any clinical summary, is ordinarily treated as incomplete, since a case manager cannot assess the described treatment against the terms of this Policy without knowing what the treatment is expected to involve.

Where a planned Confinement is extended once it has begun, for example because the treating Physician recommends further inpatient care beyond what was described at pre-authorisation, the treating Physician or the Hospital notifies the case manager assigned to the file as soon as the need for the extension becomes apparent, rather than waiting until the point of discharge. A case manager who receives such a notification aims to confirm, within one business day, whether the extended period of Confinement falls within the cover already pre-authorised or whether it requires a fresh assessment; this is treated as a priority alongside new requests rather than as a lower priority because the Confinement has already begun.

Annex A.3 The Role of Case Managers

A case manager is a member of the Company's case management team with a nursing or other clinical background, whose role is to coordinate a request for pre-authorisation and, where a Confinement follows, to remain a point of contact for its duration. A case manager is distinct from the claims assessor

who later reviews the completed claim form and the supporting documents described in the Claims Guide.

A case manager is not the Insured Person's treating Physician and does not direct the course of treatment; clinical decisions about what treatment is given remain a matter between the Insured Person and the treating Physician. The case manager's role is limited to assessing what has been proposed against the terms of this Policy and to helping the practical arrangements around a Confinement run smoothly; a case manager who has a question about the clinical appropriateness of a proposed treatment raises it with the treating Physician as a query, not as an instruction, and a decision not to pre-authorise a particular item does not prevent the treating Physician from proceeding with it if that is considered clinically necessary.

1. Reviewing the clinical information submitted with a request for pre-authorisation against the terms, limits and exclusions of this Policy.
2. Liaising with the treating Physician, with the Insured Person's consent, to clarify a diagnosis, a proposed procedure, or an expected length of stay.
3. Coordinating with the admission or finance office of a Hospital that participates in the direct settlement arrangements described in Annex A.4 below, so that the arrangements can be set up before admission where this is practical.
4. Monitoring the length of a Confinement against the course of treatment that was described at the time of pre-authorisation, and discussing with the treating Physician any departure from that course of treatment that may affect the benefit payable.
5. Acting as a single point of contact during a Confinement so that the Insured Person, or a family member, has one person to approach with a question about how the pre-authorisation is being applied.
6. Referring a request to a more senior clinical reviewer where the case manager considers that the request raises a question of complexity, or a question about how an exclusion applies, before a decision is issued.

Clinical information handled by a case manager is treated in the same way as any other health information collected in connection with a claim, and is subject to the purposes of collection, the restrictions on further transfer, and the safeguards for sensitive data described in the Personal Information Collection Statement elsewhere in this bundle. A case manager does not disclose clinical information to a person who is not the Insured Person, or to whom the Insured Person has not consented, other than to the extent described in that statement.

Where an Insured Person or a family member is more comfortable communicating in a language other than the one in which a case manager first responds, this can be requested, and the case management team will arrange for a case manager who can continue in that language where one is available, or will arrange interpretation for a telephone discussion where it is not; a request of this kind does not delay the target turnaround times described in Annex A.2 above, since the clinical review itself proceeds on the written information already provided regardless of the language used for verbal contact.

1. A case manager working on a routine request is available during the Company's normal business hours, and a request received outside those hours is treated as received at the start of the next business day for the purpose of the target turnaround.
2. A case manager working on an admission already in progress remains contactable for the duration of that Confinement through the reference number given in the original decision, so that the Insured Person or the Hospital does not need to start a new enquiry from the beginning.

3. Where the case manager originally assigned to a file is unavailable, for example because of leave, another case manager takes over the file and has access to the same clinical record and correspondence, so that continuity is maintained without the Insured Person needing to repeat information already given.

Where an Insured Person or a treating Physician disagrees with the clinical basis for a case manager's assessment, rather than with the administrative handling of the request, the matter can be raised with the senior case manager who supervises the team before it is taken up as a formal appeal under Annex A.5 below; this informal step is offered because a number of disagreements arise from a misunderstanding of the clinical information already on file rather than from a genuine difference of clinical opinion, and can often be resolved more quickly this way. Using this informal step does not use up, or shorten, the time available to bring a formal appeal if the disagreement is not resolved.

Annex A.4 Direct Settlement Arrangements with Providers

Where a course of treatment has been pre-authorised, and the Hospital or day surgery centre providing the treatment participates in the Company's direct settlement arrangements, the provider may bill the Company directly for the eligible portion of the account, rather than requiring the Insured Person to pay the account and later submit a claim for reimbursement. This is an administrative convenience: it changes who receives payment, but it does not change the benefits, limits or exclusions that apply under this Policy.

1. Before admission, the provider confirms the pre-authorisation reference with the case manager and checks the extent of the cover that has been confirmed.
2. During the Confinement, the case manager may discuss the account informally with the provider's finance office where the length of stay or the course of treatment develops differently from what was described at pre-authorisation.
3. On discharge, the provider submits the itemised account directly to the Company rather than to the Insured Person.
4. The Company reviews the account against the benefits and limits that apply, and settles the eligible portion directly with the provider.
5. Where part of the account falls outside the benefits or limits that apply, or relates to an excluded item, that part remains payable by the Insured Person directly to the provider.
6. Where a provider does not participate in these arrangements, or where treatment was not pre-authorised, the Insured Person pays the account and submits a claim for reimbursement in the usual way described in the Claims Guide.

A direct settlement arrangement is described here only in principle. It depends on an agreement between the Company and the individual provider, on the provider's own billing practices, and on the case manager's assessment at the time; it is not a separate benefit and does not entitle an Insured Person to demand that a particular provider participate. Details of providers that currently participate are not reproduced in this Supplement and should be confirmed, where relevant, through the channels described in the Claims Guide.

An Insured Person is not disadvantaged, in terms of the benefit ultimately payable, by choosing a provider that does not participate in direct settlement; the same Benefit Limits and the same exclusions apply either way, and the only practical difference is who pays the account in the first instance and, correspondingly, whether the Insured Person needs to submit a reimbursement claim afterwards. An Insured Person who would prefer to pay the account directly and claim reimbursement, even where a provider does participate in direct settlement, may ask the case manager to arrange this instead, and the request is accommodated wherever the provider's own billing arrangements allow it.

A provider that participates in direct settlement for one Insured Person's treatment is not thereby obliged to participate for a different Insured Person, and a provider may withdraw from the arrangement, or vary the terms on which it takes part, on notice to the Company. Where this happens after a pre-authorisation has already been issued for a planned admission, the case manager tells the Insured Person as soon as the change is known, so that the Insured Person can decide whether to proceed on the ordinary reimbursement basis instead.

Some providers ask for a deposit before admission, to be held against the account until it is settled; where direct settlement applies, the deposit is refunded to the Insured Person once the provider has been paid directly for the eligible portion of the account, and only the portion of the deposit that corresponds to a non-covered or excluded item is retained by the provider. The Company is not a party to the deposit arrangement itself, which is a matter between the Insured Person and the provider, but a case manager will raise a query with the provider on the Insured Person's behalf where a refund is unreasonably delayed after direct settlement has been completed.

Settlement between the Company and a provider under this Annex is made in the same currency as the main Policy, in the same way as a reimbursement claim under Part E of the Policy Wording. A dispute between the Company and a provider about the amount properly payable for an item on the account, for example a disagreement about whether a particular charge reflects the Reasonable and Customary level for that item, is resolved between the Company and the provider directly and does not reduce the benefit otherwise payable to the Insured Person; the Insured Person is not asked to make up a shortfall that arises from such a dispute, as distinct from a shortfall that arises because part of the account falls outside the benefits or limits that apply.

Where the treatment given during a Confinement turns out, once the account is reviewed, to exceed the Benefit Limit that was estimated at pre-authorisation, for example because the Confinement lasted longer than expected, the case manager tells the provider and the Insured Person as soon as this becomes apparent, rather than only at the point the account is finally settled. Direct settlement continues for the portion of the account that remains within the applicable limit, and the Insured Person is asked to settle the remaining portion with the provider directly, in the same way as for any other item that falls outside the benefits or limits that apply.

Annex A.5 Appeals Against a Pre-authorisation Decision

A decision on a request for pre-authorisation, including a decision to approve part only of a request, to defer a decision pending further information, or not to pre-authorise the treatment described, may be appealed separately from the general complaints procedure described elsewhere in this bundle. An appeal under this Annex is a clinical review; a complaint under that procedure is concerned instead with how the Insured Person's request was handled as a matter of service.

1. A written appeal is submitted within 30 days of the date of the pre-authorisation decision, together with any further clinical information from the treating Physician that supports the appeal.
2. The appeal is reviewed by a Physician engaged by the Company who was not involved in the original decision and who has relevant clinical experience.
3. The reviewing Physician may request further information from the treating Physician, and may, with the Insured Person's consent, discuss the case directly with the treating Physician.
4. A response is ordinarily given within 10 business days of all requested information being received; where the appeal concerns treatment that cannot reasonably wait, the case manager discusses an interim position with the treating Physician while the review continues.

5. Where the Insured Person remains dissatisfied after this review, the matter may be taken up under the complaints procedure described elsewhere in this bundle, which considers whether the review itself was carried out fairly and in accordance with this Annex.

Reference: [as shown on the acknowledgement] Date: [date of issue]

Dear Policyholder,

We write to confirm that we have received your request for a review of a pre-authorisation decision issued by our case management team. Your request has been passed to a Physician who was not involved in the original decision, and who will consider the additional clinical information you have provided together with the material already on file.

We aim to complete this review within the timescale described in Annex A.5 of this bundle. If the reviewing Physician needs further clinical information, we will write to the treating Physician directly, and we will let you know that we have done so.

This review relates only to the pre-authorisation decision. It does not affect any right you may have to raise a separate service complaint under the procedure described elsewhere in this bundle, and it does not require you to postpone treatment that your Physician considers necessary.

Yours faithfully,

Karen Yip

Case Management Officer

Reference: [as shown on the acknowledgement] Date: [date of issue]

Dear Policyholder,

Further to our letter acknowledging your request for a review, the reviewing Physician has now completed the clinical assessment described in Annex A.5 of this bundle.

Where the review supports the treatment plan submitted by your Physician, we will confirm the revised pre-authorisation reference and validity period in a separate notice, and, where a provider participates in direct settlement, we will update that provider so that the arrangement described in Annex A.4 can proceed without further action on your part.

Where the review does not support part or all of the original request, we will explain the clinical basis for that outcome in plain terms, identify which part of the request is affected, and set out how the position may change if further clinical information becomes available.

A pre-authorisation review is separate from the assessment of a claim once treatment has been completed and the supporting documents have been submitted, and this letter does not itself determine whether a benefit is payable.

Yours faithfully,

Karen Yip

Case Management Officer

An appeal under this Annex does not require an Insured Person to delay treatment that the treating Physician considers necessary. Where treatment proceeds before the appeal is concluded, the position described in Annex A.1 above applies: the treatment is reviewed on its clinical merits once the claim and its supporting documents are submitted, in the same way as any other course of treatment for which pre-authorisation was not obtained in advance.

An appeal is considered on the clinical information submitted with it; a bare statement of disagreement with the original decision, without further clinical information, is unlikely to lead to a different outcome unless the reviewing Physician identifies a point in the original assessment that should be reconsidered on the material already available. Supporting information from the treating Physician is most useful where it explains why the treatment proposed is expected to address the diagnosis more effectively, more safely, or with a materially better prospect of recovery than an alternative the original decision may have contemplated, rather than repeating the request in the same terms as before.

Where the reviewing Physician recommends that the Company obtain a further, independent medical opinion before the appeal is decided, the cost of that opinion is borne by the Company, and the Insured Person is not asked to attend an examination for this purpose without being told in advance why it has been requested and what it is expected to address. A record of the appeal, the information considered, and the outcome is kept with the case management file and may be referred to again if a related question arises in a later claim for the same or a connected condition.

Time spent on an appeal under this Annex does not extend the time limit within which a claim must be submitted once treatment has been completed; an Insured Person who is waiting for the outcome of an appeal, but whose treatment has already been given, should still submit the claim form and the supporting documents within the usual time limit, noting on the form that a related appeal is in progress, so that the claim is not treated as late while the appeal is being considered separately.

A pre-authorisation review conducted under this Annex is carried out on the clinical information available at the time and does not bind the Company's final assessment of a claim, which is made under Part E of the Policy Wording once treatment has been completed.

